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Question **1**

Not answered

Marked out of 5.00

Isolation
Regression
Denial
Denigration
Suppression
Projection
Projective identification
Splitting
Idealization
Turning against the self

HiY Assessment CXEMI001

Self harm and defence mechanisms

A 22-year-old woman was admitted to the psychiatric ward with history of depression and recurrent attempts to harm herself using razors. She is complaining of chronic feelings of emptiness and constant fear of abandonment. She has no friends and is not in any relationship. From the list above, match the most likely defence mechanisms being described below. Each option can be used once, more than once or not at all.

1. She says that the consultant who wants her to stay on the ward is an excellent professional, but the nurses are being irrational and insist that she should be discharged. (Choose THREE)

2. Her arms and legs show superficial lacerations of self-cutting. (Choose ONE)

3. She has had psychotherapy in the past. The trainee therapist started feeling depressed and went off sick after seeing her for a few sessions. (Choose ONE)

Explanation:

Idealisation involves perceiving others as ideal in order to avoid conflicting feelings about them. Humans find it less anxiety-provoking to avoid ambivalence and grey areas within people. The converse of idealisation is denigration where only bad is seen within others. Functionally, patients with borderline personality disorder distort their relationships by considering each person to be either all good or all bad. They see people as either nurturing attachment figures or as hateful, sadistic figures who deprive them of security needs and threaten them with abandonment whenever they feel dependent. As a result of this splitting, the good person is idealised and the bad person devalued

Passive aggression is aggression toward an object expressed indirectly and ineffectively through passivity, masochism, and turning against the self. In extremes, the resentment is not just expressed indirectly toward the other, but in fact, is turned 180 degrees around toward the self (turning against the self) to get at the other.

Projective identification involves behaving towards another in a manner that causes them to take one's on own internal unacceptable ideas and impulses. It should not be confused with projection. Whereas during projection an individual with a certain emotion might perceive someone else as feeling the same way, in projective identification the individual causes the other to feel that emotion (e.g. an angry child

behaves in such a way that his mother becomes angry with him).

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. P. 892.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 564, 1066.

Mariagrazia Di Giuseppe J. The Hierarchy of Defense Mechanisms: Assessing Defensive Functioning With the Defense Mechanisms Rating Scales Q-Sort. Frontiers in Psychology. 2021;12:1-23.

The correct answer is:

She tells that the consultant who wants her to stay on the ward is an excellent professional, but the nurses are being irrational and insist that she should be discharged (Choose THREE) - Splitting, Denigration, Idealisation

Her arms and legs show superficial lacerations of self-cutting (Choose ONE) - Turning against the self

She has had psychotherapy in the past. The trainee therapist started feeling depressed, helpless and went on protracted medical leave after seeing her for a few sessions (Choose ONE) - Projective identification

Question **2**

Not answered

Marked out of 8.00

Disorders and Freudian defences

Match the following condition with the most likely defence mechanisms provided in the list. Each option can be used once, more than once or not at all.

Obsessive-compulsive disorder	Choose...
Impulse control disorders	Choose...
Grief	Choose...
Suicide	Choose...
Fugue	Choose...

Choose...
Denial
Introjection
Repression
Turning against the self
Dissociation
Idealisation
Isolation of affect
Projection
Acting out

Explanation:

Isolation of affect is a defence mechanism in which emotion is detached from an idea and rendered unconscious, leaving the idea bland and emotionally flat. It is especially important in obsessive-compulsive disorder. In non-disordered people it most often occurs following a traumatic experience. The mechanisms by which the detachment is effected include repetitive intrusive thoughts and ritualistic behaviour.

In acting out, patients directly express unconscious wishes or conflicts through action to avoid being conscious of either the accompanying idea or the affect. Tantrums, apparently motiveless assaults, and pleasureless promiscuity are typical examples.

The introjection of a loved one involves the internalisation of characteristics of the object with the goal of ensuring closeness to and constant presence of the object. If the object is lost, introjection nullifies or negates the loss by taking on the characteristics of the object, thus in a sense internally preserving the object.

Passive aggression is aggression toward an object expressed indirectly and ineffectively through passivity, masochism, and turning against the self. In extremes, the resentment is not just expressed indirectly toward the other, but in fact, is turned 180 degrees around toward the self (turning against the self) to get at the other.

Dissociation is a temporary but drastic modification of character or sense of personal identity to avoid

emotional distress. It includes fugue states and conversion reactions.

Ref: Colman AM. Oxford Dictionary of Psychology, 4th Edition. 2015. p. 394.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 577, 1066-1067.

Mariagrazia Di Giuseppe J. The Hierarchy of Defense Mechanisms: Assessing Defensive Functioning With the Defense Mechanisms Rating Scales Q-Sort. Frontiers in Psychology. 2021;12:1-23.

The correct answer is: Obsessive-compulsive disorder → Isolation of affect, Impulse control disorders → Acting out, Grief → Introjection, Suicide → Turning against the self, Fugue → Dissociation

Question **3**

Not answered

Marked out of 5.00

Defence mechanisms in clinical practice

From the list given here, match the most likely defence mechanisms being described below

A 45-year-old man was frustrated by problems at work. He then became violent at home towards his wife and children. Choose...

A 65-year-old woman has been diagnosed with breast cancer. She is refusing to accept that she has cancer. Choose...

A 25-year-old man says 'I find gardening a good outlet for my pent-up aggression.' Choose...

Since the death of her long-term partner, a 45-year-old woman has been acting like a child. She is losing her temper and is eating excessively for comfort. Choose...

A 55-year old woman was talking about her car accident, in which she lost her partner, without any display of emotions. Choose...

Choose...
Isolation of affect
Regression
Splitting
Denial
Projection
Repression
Displacement
Sublimation
Reaction formation
Identification with the aggressor

Explanation:

Displacement is the process by which interest and/or emotion is shifted from one object onto another less threatening one. For example, the person who is having problems at work may displace the anger felt for their boss onto their family by displaying irritability and moodiness at home.

Denial is defined as the expressed refusal to recognise a threatening reality. It is of relevance especially to those with serious physical illnesses, where the patient denies being told of the presence of illness in themselves or their loved ones. It may persist despite constant reiteration of the facts.

Sublimation is the gratification of an impulse whose goal is retained but whose aim or object is changed from a socially objectionable one to a socially valued one. Sublimation of aggressive impulses takes place through pleasurable games, sports, and hobbies. Unlike neurotic defences, sublimation allows instincts to be channeled rather than damned up or diverted. Thus, in sublimation, feelings are acknowledged, modified, and directed toward a relatively significant person or goal so that modest instinctual satisfaction results.

Regression involves responding to stress by reverting to a level of functioning of a previous maturational point (e.g. a teenager sucking their thumb around exam time).

Isolation of affect is a defence mechanism in which emotion is detached from an idea and rendered unconscious, leaving the idea bland and emotionally flat. It is especially important in obsessive-compulsive disorder. In non-disordered people it most often occurs following a traumatic experience.

Ref: Casey P, Kelly B. Fish's clinical pathology: Signs and symptoms in psychiatry, 4th Edition. 201 p. 129.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 1067.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 892.

Colman AM. Oxford Dictionary of Psychology, 4th Edition. 2015. p. 394.

The correct answer is: A 45-year-old man was frustrated by problems at work. He then became violent at home towards his wife and children. → Displacement, A 65-year-old woman has been diagnosed with breast cancer. She is refusing to accept that she has cancer. → Denial, A 25-year-old man says 'I find gardening a good outlet for my pent-up aggression.' → Sublimation, Since the death of her long-term partner, a 45-year-old woman has been acting like a child. She is losing her temper and is eating excessively for comfort. → Regression, A 55-year old woman was talking about her car accident, in which she lost her partner, without any display of emotions. → Isolation of affect

Question **4**

Not answered

Marked out of 5.00

Choose...
Denial
Reaction formation
Identification
Projection
Displacement
Repression
Sublimation
Suppression
Regression
Rationalisation

Defence mechanisms in daily life

From the list above, match the most likely defence mechanisms being described below

When going for an interview, a 27-year old man makes a voluntary decision not to think about an argument with his girlfriend that he had earlier. Choose...

A 45-year-old man hit his wife, then says that he is cruel 'for [her] own sake'. 'I only did it because I love you.' Choose...

A 16-year-old girl hates her classmate. However, she is very polite and goes out of her way to be nice to him. Choose...

A 24-year-old woman with a personality disorder says that she has never done anyone any harm and that her distress is the fault of others, whom she blames. Choose...

A 5-year-old boy started to behave like his father and seemed to have acquired the mannerisms and qualities of his dad. Choose...

Explanation:

Suppression is the conscious or semiconscious decision to postpone attention to a conscious impulse or conflict.

Rationalisation is a justification of attitudes, beliefs, or behaviour that might otherwise be unacceptable by an incorrect application of justifying reasons or the invention of a convincing fallacy.

Reaction formation involves externally expressing attitudes and behaviours that are the opposite of the unacceptable internal impulses (e.g. being extra polite to a person to avoid expressing anger towards them).

Projection is a primitive or immature defense mechanism that involves attributing one's own internal unacceptable ideas and impulses to an external target, such as another individual, and reacting accordingly to them. It includes severe prejudice, rejection of intimacy through suspiciousness, hypervigilance to external danger, and injustice collecting.

Identification involves taking on the characteristics, feelings, and/or behaviours of someone else as one's own. It differs from introjection in a similar manner to the way projective identification differs from projection. Whereas in introjection one may perceive themselves as being like someone else, in identification one may actually feel the same way someone else does and become more like the other person.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 1066-1067.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 892-

893.

The correct answer is: When going for an interview, a 27-year old man makes a voluntary decision not to think about an argument with his girlfriend that he had earlier. → Suppression, A 45-year-old man hit his wife, then says that he is cruel 'for [her] own sake'. 'I only did it because I love you.' → Rationalisation, A 16-year-old girl hates her classmate. However, she is very polite and goes out of her way to be nice to him. → Reaction formation, A 24-year-old woman with a personality disorder says that she has never done anyone any harm and that her distress is the fault of others, whom she blames. → Projection, A 5-year-old boy started to behave like his father and seemed to have acquired the mannerisms and qualities of his dad. → Identification

Question **5**

Not answered

Marked out of 4.00

Defence mechanisms - definitions

For each of the definitions given below, identify the most likely defence mechanisms from the list provided:

Choose...
Repression
Sublimation
Denial
Projection
Undoing
Acting out
Isolation of affect
Splitting
Reaction formation
Identification with the aggressor

Unconsciously motivated acts which magically or symbolically counteract unacceptable thoughts, impulses or acts. — Choose...

Unconscious viewing of self or others as good or bad without considering the whole range of qualities. — Choose...

Unconscious gradual channelization of unacceptable infantile impulses into personally satisfying and socially valuable behaviour patterns. — Choose...

Separation of the idea of an unconscious impulse from its appropriate affect thus allowing only the idea and not the associated affect to enter awareness. — Choose...

Explanation:

Undoing is found when performing an action that has the effect of unconsciously ‘cancelling out’ an unacceptable internal impulse or previous experience. The action symbolises the opposite outcome of the impulse or experience.

Splitting occurs when people, both past and present, are divided into their polar opposites. Thus they are regarded as either perfect or deeply flawed, exclusively nurturing or rejecting.

Sublimation is the gratification of an impulse who goal is retained but whose aim or object is changed from a socially objectionable one to a socially valued one. Sublimation of aggressive impulses takes place through pleasurable games, sports, and hobbies. Unlike neurotic defences, sublimation allows instincts to be channeled rather than damned up or diverted. Thus, in sublimation, feelings are acknowledged, modified, and directed toward a relatively significant person or goal so that modest instinctual satisfaction results.

Isolation of affect is a defence mechanism in which emotion is detached from an idea and rendered unconscious, leaving the idea bland and emotionally flat. It is especially important in obsessive-compulsive disorder. In non-disordered people it most often occurs following a traumatic experience.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 893

Casey P, Kelly B. Fish’s clinical pathology: Signs and symptoms in psychiatry, 4th Edition. 2019. p. 130.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 1067.

Colman AM. Oxford Dictionary of Psychology, 4th Edition. 2015. p. 394.

The correct answer is: Unconsciously motivated acts which magically or symbolically counteract

unacceptable thoughts, impulses or acts. → Undoing, Unconscious viewing of self or others as good or bad without considering the whole range of qualities. → Splitting, Unconscious gradual channelization of unacceptable infantile impulses into personally satisfying and socially valuable behaviour patterns. → Sublimation, Separation of the idea of an unconscious impulse from its appropriate affect thus allowing only the idea and not the associated affect to enter awareness. → Isolation of affect

Question **6**

Not answered

Marked out of 4.00

Neurological signs

Match the following clinical signs correctly:

A 74-year-old man presents with difficulty while turning on the bed, tremors even while at rest, stiffness of both forearm muscles and depression. — Choose...

A 24-year-old man presents with bizarre behaviour, anxiety and depression. He has a bird-like wing beating tremor. On examination, liver functions are abnormal, and a genetic disease is suspected. — Choose...

A 4-year-old boy has poor motor development in spite of excellent cognitive development. He has quite bulky muscles on inspection, and no facial dysmorphism is noted. His brother had suffered from a similar problem before he died of cardiomyopathy at the age of 16. — Choose...

A 73-year old gentleman with resting tremor, cog- wheal rigidity continues to blink with repeated glabellar taps. — Choose...

Choose...
Ash leaf macules
Argyll Robertson pupil
Tuft of hair on midline
Kayser-Fleischer ring
Shuffling gait
Chvostek's sign
Gower's sign
Milkmaid's grip
Myerson Sign
Frontal baldness

Shuffling gait: The patient exhibits features of Parkinson's disease with tremors and axial rigidity leading to difficulty while turning on a bed. Patients have short-stepped shuffling gait. The pace may vary with a tendency for the patient to accelerate i.e. progressively short (festinating gait).

Kayser-Fleischer (KF) ring is a brownish ring around the limbus of the cornea. It is best demonstrated during an ophthalmologic slit lamp examination. This patient exhibits features of Wilson's disease. KF rings (copper deposits on Descemet's membrane of the cornea) are present in 95% of patients with neurological symptoms, in 50-60% of patients without neurological symptoms and in only 10% of asymptomatic siblings. Personality disturbances, mood abnormalities, and cognitive dysfunction can be seen in Wilson's disease. A wing-beating tremor is another important feature of this condition.

Gower sign is seen in severe myopathies. It occurs when the patient attempts to stand up from the floor. Patients first sit up, then assume a quadruped position, and then climb up their legs by using their arms to push themselves up. This is an important feature of Duchenne's muscular dystrophy which is an X-linked recessive disorder. Due to muscle degeneration, patients experience difficulty when walking and breathing, and may die eventually. The incidence is 1 in 3,000. Only males are affected while females are carriers.

Myerson sign: Patients with Parkinson disease and those experiencing bilateral frontal lesions, will continue to blink with renewed glabellar taps.

Ref: Yiu, E; Kornberg, A. "Duchenne muscular dystrophy". Neurology India 2008;56(3):236-47

The correct answer is: A 74-year-old man presents with difficulty while turning on the bed, tremors even while at rest, stiffness of both forearm muscles and depression. → Shuffling gait, A 24-year-old man presents with bizarre behaviour, anxiety and depression. He has a bird-like wing beating tremor. On examination, liver functions are abnormal, and a genetic disease is suspected. → Kayser-Fleischer ring, A 4-year-old boy has poor motor development in spite of excellent cognitive development. He has quite bulky muscles on inspection, and no facial dysmorphism is noted. His brother had suffered from a similar problem before he died of cardiomyopathy at the age of 16. → Gower's sign, A 73-year old gentleman with resting tremor, cog- wheal rigidity continues to blink with repeated glabellar taps. → Myerson Sign

Question **7**

Not answered

Marked out of 6.00

Physical signs

For each of the description given below, identify the physical examination findings from the list provided.

An 18-year-old girl hides food in cupboards, refuses to sit with others while eating and has lost considerable weight over the last six months. She was a high flyer at school but recently her performance has been declining. She does not induce vomiting but exercises for 3 hours a day. — Choose...

A 22-year-old woman who has a previous history of an eating disorder attends clinic with a history of uncontrollable episodes of overeating that culminate in purging and vomiting. She has maintained reasonable weight but feels she has lost control over her eating again. — Choose...

A 43-year-old heroin user presents to the A&E demanding a strong painkiller for stomach ache. He threatens to bleed himself to death if he is not given a parenteral opioid analgesic. — Choose...

A 33-year-old man was started on clozapine three weeks ago but has developed pneumonia recently. His WBC count is normal. — Choose...

Choose...
Mask like face
Lid lag
Piloerection
Lemon stick appearance
Constricted pupil
Lanugo hair
Argyll Robertson Pupil
Xanthelasma
Goiter
Russell sign
Sialorrhea

Explanation:

Lanugo is fine hair or down, especially the woolly covering of human foetus at birth and the fluffy hair that often appears on the faces and bodies of people with anorexia nervosa.

Russell sign is comprised of skin abrasions, small lacerations, and calluses on the dorsum of the hand overlying the metacarpophalangeal and interphalangeal joints found in patients with symptoms of bulimia. It is caused by repeated contact between the incisors and the skin of the hand, which occurs during self-induced vomiting.

Piloerection is the raising of hair into an upright position as a result of contraction of smooth muscles attached to the base of hair follicles. The withdrawal syndrome from opioids can include piloerection or gooseflesh (from which derives the term ‘cold turkey’ for the abstinence syndrome).

Clozapine is well known to be causally associated with hypersalivation (sialorrhea), with excess salivary pooling in the mouth and drooling, particularly at night. Clozapine-induced hypersalivation could be potentially life-threatening, as it has been implicated as a contributory factor in the development of aspiration pneumonia.

Ref: Colman AM. Oxford Dictionary of Psychology, 4th Edition. 2015. p. 411, 577.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 117.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 292.

Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 227.

The correct answer is: An 18-year-old girl hides food in cupboards, refuses to sit with others while eating and has lost considerable weight over the last six months. She was a high flyer at school but recently her performance has been declining. She does not induce vomiting but exercises for 3 hours a day. → Lanugo hair, A 22-year-old woman who has a previous history of an eating disorder attends clinic with a history of uncontrollable episodes of overeating that culminate in purging and vomiting. She has maintained reasonable weight but feels she has lost control over her eating again. → Russell sign, A 43-year-old heroin user presents to the A&E demanding a strong painkiller for stomach ache.

He threatens to bleed himself to death if he is not given a parenteral opioid analgesic. → Piloerection, A 33-year-old man was started on clozapine three weeks ago but has developed pneumonia recently. His WBC count is normal. → Sialorrhea

Question **8**

Not answered

Marked out of 5.00

General examination findings

For each of the following clinical situations, identify the most likely findings on physical examination

A 45-year-old homosexual drug abuser presenting to A&E with a history of mania and weight loss — Choose...

A 67-year-old woman with 20-year history of diabetes and depression — Choose...

A 57-year-old gentleman with chronic schizophrenia on maintenance treatment with olanzapine for 12 years — Choose...

A 45-year-old woman presenting repeatedly to A& E with history of chronic abdominal pain and self-inflicted injuries — Choose...

A 35-year-old gentleman with a history of heavy alcohol use — Choose...

Choose...
Argyll-Robertson pupil
Generalised lymph node enlargement
Xanthelasma
Goitre
Checkerboard abdomen
Jaundice
Kayser-Fleischer ring
Sialorrhoea
Dilated pupil
Lanugo hair

This is a case of HIV illness, and generalised lymph node enlargement is a common finding.

The more common cause of Argyll-Robertson pupil is diabetes mellitus, and the other common cause is neurosyphilis, which is rare to see these days.

Xanthelasma is due to lipid accumulation, which is often related to long-term treatment with antipsychotic medications such as olanzapine.

Multiple surgical scars are often seen in patients with factitious disorder leading to a checker board abdomen.

Heavy alcohol use can cause impairment of liver functions liver leading to jaundice, hepatitis, fatty liver and cirrhosis.

The correct answer is: A 45-year-old homosexual drug abuser presenting to A&E with a history of mania and weight loss → Generalised lymph node enlargement, A 67-year-old woman with 20-year history of diabetes and depression → Argyll-Robertson pupil, A 57-year-old gentleman with chronic schizophrenia on maintenance treatment with olanzapine for 12 years → Xanthelasma, A 45-year-old woman presenting repeatedly to A& E with history of chronic abdominal pain and self-inflicted injuries → Checkerboard abdomen, A 35-year-old gentleman with a history of heavy alcohol use → Jaundice

Question **9**

Not answered

Marked out of 5.00

Flag question

Interview techniques

For each of the following statements given below, identify the interview technique used;

What brings you to the clinic today? — Choose...

'You said you are feeling low in mood ever since you can remember. When do you feel most depressed?' — Choose...

The patient said, 'I was having bad dreams last night'. The doctor said

Choose...
Closed question
Summation
Closed questions
Open-ended question
Confrontation
Facilitation
Repetition
Redirection
Restatement
Clarification

'So, you are getting disturbed by the dreams you have'. Choose...

'So from what you told so far, you are anxious for the last three months and not sleeping well, and your job is at risk. Right?' Choose...

The patient said, 'I was having bad dreams last night'. The doctor said 'So, you were having bad dreams last night'. Choose...

Explanation:

Open-ended questions are difficult to answer with one word or a short phrase, even if the interviewee is moderately guarded or moderately resistant. It is virtually impossible to respond to these questions with a simple ‘yes’ or ‘no’. Questions that provide or imply possible answers, or ask for specific items, places, dates, numbers, or names, are never viewed as open-ended because they limit the freedom of answer choice. In patients who are easily engaged, open-ended questions should tend to produce relatively large quantities of speech.

Clarification of what the patient has said can sometimes lead to unrecognised issues or psychopathology.

Effective communication can also involve restatement and repetition. Repetition is repeating the exact words of the patient. Restatement is similar to repetition, but the phrases are rearranged.

Periodically during an interview, it is helpful to summarise what has been identified about a certain topic (‘summation’). This provides the opportunity for the patient to clarify or modify the psychiatrist’s understanding and possibly add new material.

Ref: Shea SC. Psychiatric interviewing: The art of understanding, 3rd Edition. 2016. p. 795.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 14-15.

The correct answer is: What brings you to the clinic today? → Open-ended question, 'You said you are feeling low in mood ever since you can remember. When do you feel most depressed?' → Clarification, The patient said, 'I was having bad dreams last night'. The doctor said 'So, you are getting disturbed by the dreams you have'. → Restatement, 'So from what you told so far, you are anxious for the last three months and not sleeping well, and your job is at risk. Right?' → Summation, The patient said, 'I was having bad dreams last night'. The doctor said 'So, you were having bad dreams last night'. → Repetition

Question **10**

Not answered

Marked out of 5.00

Psychiatric Interview Skills

For each of the following statements, identify the interview technique used;

Choose...
Transition
Closed question
Limit setting
Redirection
Summation
Confrontation
Clarification
Interpretation
Question rephrasing
Self-revelation

You seem not to have gained any weight in the last three months. Is it possible that your eating has been poor again? Choose...

I am going to interrupt you as there are few important things we need to cover today. Choose...

Did you sleep well last night? Choose...

Do you like Sir Cliff Richards? I was a mad fan while at school. Choose...

You mentioned that your mother is a receptionist. What about yourself? What job do you do? Choose...

Confrontation is pointing out to a patient something to which the doctor thinks the patient is missing or denying. It must be done in a respectful way. The aim is to help patients face a difficult aspect rather than dismissing patients by showing a negative aspect.

Limit setting is useful to manage time pressure, especially in garrulous patients. It should be used cautiously, overuse may detach patient from the doctor. The motive is to use time effectively.

Closed questions: When, where, how many, which and what questions. Answers can only be 'yes or no', in most occasions. When clubbed with non-facilitative gestures, can be detrimental to interview process.
Stating a presumption followed by tags can be very directive. It is better avoided in early parts of an interview as they can produce prescribed answers lacking in detail. Also, avoid in highly suggestible patients. A good technique is to start with open questions and moving to closed questions by the end of an interview. Closed questions are useful to rule out less likely symptoms.

Self-Revelation is limited, discreet self-disclosure by physicians. It helps the physician to feel at-ease sometimes. Excessive self-revelation is a boundary violation.

The transition is moving from one to another topic - this is a special skill and preferably must be done as smoothly as possible to keep the patient interested. Smooth transitions - uses the cue off something the patient just said.
Ref: Adapted from Kay J & Tasman A. Essentials of Psychiatry, 2nd edition, 2006. John Wiley & Sons, Ltd.

The correct answer is: You seem not to have gained any weight in the last three months. Is it possible that your eating has been poor again? → Confrontation, I am going to interrupt you as there are few important things we need to cover today. → Limit setting, Did you sleep well last night? → Closed question, Do you like Sir Cliff Richards? I was a mad fan while at school. → Self-revelation, You mentioned that your mother is a receptionist. What about yourself? What job do you do? → Transition

Question **11**

Not answered

Marked out of

Investigating alcoholism

For the statements given below, identify from the given list the most appropriate test for patients with alcohol abuse.

Choose...
Gamma glutamyl transpeptidase
Mean corpuscular volume
Mean corpuscular hemoglobin concentration
Triglycerides
Uric acid
Aspartate aminotransferase
Serotonin deficient transferring
AUDIT
Alanine aminotransferase
Carbohydrate-deficient transferrin

This is a sensitive marker of alcohol abuse	Choose...
This test accurately predicts recent alcohol abuse and can be useful in detecting relapse	Choose...
This test is more specific and is less useful in detecting relapse	Choose...
This is the best screening tool in primary care	Choose...

Gamma-glutamyl transpeptidase (GGT) is a sensitive marker of alcohol abuse. Levels return to normal 2-4 weeks after abstinence and, therefore, helpful in identifying those who have relapsed after treatment.

Mean corpuscular volume (MCV): Elevated MCV is more specific but less sensitive than GGT for alcohol abuse. Due to the long lifespan of RBCs, MCVs may not return to normal for 2-4 months after abstinence and so is less useful in detecting relapse.

Carbohydrate-deficient transferrin (CDT) accurately predicts recent alcohol abuse and can be useful in detecting relapse. No single lab test can dependably diagnose alcohol abuse. Carbohydrate-deficient transferrin is the single most specific and sensitive test for detecting heavy alcohol use over last ten days. But due to a high degree of intersubject variability it is best to compare CDT levels with patient's

baseline.

In primary care, AUDIT is the best screening tool (Alcohol Use Disorders Identification Test) This is a 10-item questionnaire, covering quantity, frequency, inability to control drinking, withdrawal relief, loss of memory, injury and concern by others. A score of 8 or higher indicates drinking to a degree that is harmful or hazardous, whereas a score of 13 or more in women and 15 or more in men is indicative of dependent drinking. It is a very useful and widely used scale, recommended by WHO for primary care use.

The correct answer is: This is a sensitive marker of alcohol abuse → Gamma glutamyl transpeptidase, This test accurately predicts recent alcohol abuse and can be useful in detecting relapse → Carbohydrate-deficient transferrin, This test is more specific and is less useful in detecting relapse → Mean corpuscular volume, This is the best screening tool in primary care → AUDIT

Question **12**

Not answered

Marked out of 6.00

Flag question

Appropriate use of rating scales

Identify the most appropriate rating scale for the following conditions.

Mrs.X suffers from OCD. She has recently delivered a baby. The health visitor would like to screen her for depression

Choose...

Choose...
CATEGO
Barnes Akathisia Scale
Present state examination
General health questionnaire
Edinburgh postnatal depression scale
Montgomery and Asberg depression rating scale, Hamilton depression rating scale
Abnormal involuntary movements scale
Hospital anxiety and depression scale
Simpson-Angus scale
Beck depression inventory

Mr.Y is taking a high dose of haloperidol. You wish to monitor the extent of parkinsonian side effects

Choose...

Mr. Z is a new trainee in psychiatry who would like to interview a patient in a standardized manner, but still retaining the features of a clinical examination

Choose...

Mr. A is a GP registrar who would like to use a computer programme to generate psychiatric diagnosis

Choose...

Mrs. P has been

diagnosed with postnatal depression and has been commenced on antidepressants. You wish to monitor her response to treatment (Choose TWO)

Choose...

Edinburgh Postnatal Depression Scale was specially developed for assessing postpartum depression and relies much less on somatic questions than traditional depression scales. Questions on the Edinburgh scale (10 items, can be self or clinician-rated) are framed within the "past seven days", and the response format is frequency-based. Routine use of EPDS during the postpartum period has been shown to increase the detection of postpartum depression compared with usual care.

The present state examination is clinician administered semi-structured clinical interview, which provides a clinical diagnosis.

CATEGO- it is a computer programme for processing data from the schedules of clinical assessment in neuropsychiatry.

Both Montgomery-Asberg depression rating scale and Hamilton rating scale for depression are useful to measure the change in depression, especially during treatment trials. These scales are helpful to monitor response to treatment.

The correct answer is: Mrs.X suffers from OCD. She has recently delivered a baby. The health visitor would like to screen her for depression → Edinburgh postnatal depression scale, Mr.Y is taking a high dose of haloperidol. You wish to monitor the extent of parkinsonian side effects → Simpson-Angus scale, Mr. Z is a new trainee in psychiatry who would like to interview a patient in a standardized manner, but still retaining the features of a clinical examination → Present state examination, Mr. A is a GP registrar who would like to use a computer programme to generate psychiatric diagnosis → CATEGO, Mrs. P has been diagnosed with postnatal depression and has been commenced on antidepressants. You wish to monitor her response to treatment (Choose TWO) → Montgomery and Asberg depression rating scale, Hamilton depression rating scale

Question **13**

Not answered

Marked out of 5.00

Rating scales - descriptions

For each description given below, select the relevant rating scale used for depressive disorders from the list provided.

Choose...
Clinical Global impressions scale (CGI)
Yale-Brown obsessive compulsive scales (YBOCS))
Young mania rating scale
General health questionnaire (GHQ)
Positive and negative symptoms scale (PANSS)
Zung depression rating scale
Beck depression inventory (BDI)
Montgomery Asberg depression rating scale (MADRS)
Hospital anxiety and depression scale (HADS)
Brief psychiatric rating scale (BPRS)
Hamilton depression scale (Ham D)

Self-rating, 20 items scale used in depression research, insensitive to change

Choose...

Observer-rated, ten items rated on a four-point scale, used in depression research and is useful to measure change in depression especially during treatment trials

Choose...

Observer-rated, 17 items, widely used in depression research to assess the severity of depressive syndrome

Choose...

Self-rated, 14 items, widely used in anxiety and depression — Choose...

A self-rated questionnaire containing 21 statements with four possible responses for each, one of which is chosen as best describing the symptom at the time. — Choose...

Beck Depression Inventory is a self-rated questionnaire containing 21 statements with four possible responses for each. The maximum score is 63; BDI lacks discriminatory power among the very severely ill and in addition, BDI includes more psychological than somatic factors.

Montgomery Asberg depression rating scale (MADRS) is a 10-item observer-rated scale, which is very sensitive to measure the change in depression scores especially during treatment trials.

Hamilton depression scale (HamD) is a 17-item rating scale for depressive illness. It's not a diagnostic instrument and is used to measure changes (e.g. as a result of drug treatment). The 17 items are scored according to severity, producing the total score.

Hospital Anxiety and Depression Scale, when used with a cut-off score of ≥8 , offers optimal sensitivity and specificity to detect cases among unselected patients in primary care. Hospital Anxiety and Depression Scale (HADS) and Patient Health Questionnaire (PHQ-9) are self rated scales that serve as useful screening tools for non-psychiatric clinicians.

Zung self-rating scale has 20 items, avoids imbalance towards psychological factors seen in Beck's and is insensitive to change

The correct answer is: Self-rating, 20 items scale used in depression research, insensitive to change → Zung depression rating scale, Observer-rated, ten items rated on a four-point scale, used in depression research and is useful to measure change in depression especially during treatment trials → Montgomery Asberg depression rating scale (MADRS), Observer-rated, 17 items, widely used in depression research to assess the severity of depressive syndrome → Hamilton depression scale (Ham D), Self-rated, 14 items, widely used in anxiety and depression → Hospital anxiety and depression scale (HADS), A self-rated questionnaire containing 21 statements with four possible responses for each, one of which is chosen as best describing the symptom at the time. → Beck depression inventory (BDI)

Question **14**

Not answered

Marked out of 6.00

Flag question

Neuroimaging tests in psychiatry

Identify 2 common neuroimaging findings for the following psychiatric conditions

A 22-year-old gentleman with persecutory delusions and 3rd person auditory hallucinations — Choose...

Choose...
Increased frontal lobe volumes
Increased blood flow in parieto-temporal areas in SPECT
Loss of white matter in the dorsolateral pre-frontal cortex
Ventricular enlargement, Poor activation of dorsolateral prefrontal cortex in executive tasks during f-MRI
Lower prefrontal metabolism
Progressive increase in brain volume in first few years of diagnosis
Ventricular enlargement, Loss of temporal lobe volume - especially hippocampus
Periventricular and deep white matter hyperintensities, Decreased metabolism in Prefrontal and Anterior cingulate cortex

A 29-year-old woman with low mood, anhedonia and poor sleep. — Choose...

A 77-year-old man with history of short-term memory loss and poor concentration — Choose...

The diagnosis for case 1 is most likely to be schizophrenia. Ventricular enlargement, Loss of greymatter - especially around cingulate cortex, insula and medial temporal lobe, along with fMRI evidence for poor dorsolateral pre-frontal cortex activation in executive tasks, decreased N-acetyl aspartate in prefrontal cortex in Magnetic Resonance Spectroscopy are some of the neuroimaging findings in schizophrenia.

The diagnosis for case 2 is likely to be depressive disorder. Neuroimaging in depressed patients shows periventricular and deep WM hyperintensities, subcortical hyperintensities, decreased frontal and basal ganglia volumes, decreased metabolism in the prefrontal cortex.

The diagnosis for case 3 is likely to be Alzheimer's dementia. Neuroimaging in Alzheimers disease reveals ventricular enlargement related to cortical atrophy, reduced medial temporal lobe volume (especially hippocampal atrophy) and decreased parieto-temporal fMRI activation and SPECT blood flow.

The correct answer is: A 22-year-old gentleman with persecutory delusions and 3rd person auditory hallucinations → Ventricular enlargement, Poor activation of dorsolateral prefrontal cortex in executive tasks during f-MRI, A 29-year-old woman with low mood, anhedonia and poor sleep. → Periventricular and deep white matter hyperintensities, Decreased metabolism in Prefrontal and Anterior cingulate cortex, A 77-year-old man with history of short-term memory loss and poor concentration → Ventricular enlargement, Loss of temporal lobe volume - especially hippocampus

Question **15**

Not answered

Marked out of 5.00

EEG wave forms

For each of the following waveforms, identify the frequency range.

Alpha — Choose...

Beta — Choose...

Theta — Choose...

Mu — Choose...

Delta — Choose...

Choose...
Single sharp wave
7-11 Hz
Spike waves
More than 13 Hz
8-13 Hz
Triangular waves
Less than 4 Hz
4-8 Hz

Alpha waves have a frequency of 8-13 Hz. Alpha waves are the dominant brainwave frequency when eyes are closed. Alpha disappears with anxiety, arousal, eye opening or focused attention. Beta waves (more than 13 Hz) are seen at frontal, central position in the normal waking EEG. Theta waves (4-8 Hz) are seen in wake EEG at frontotemporal area; prominent in drowsy or sleep EEG. Excessive theta in awake EEG is a sign of pathology. Delta waves (less than 4 Hz) are not seen in waking EEG. It is common in deeper stages of sleep; the presence of focal/generalized delta in awake EEG is a sign of pathology. Mu waves (7-11 Hz) occur over the motor cortex. Mu is related to motor activity, characterized by arch like waves.

The correct answer is: Alpha → 8-13 Hz, Beta → More than 13 Hz, Theta → 4-8 Hz, Mu → 7-11 Hz, Delta → Less than 4 Hz

Question **16**

Not answered

Marked out of 5.00

Agnosias & Apraxias

Find one term each to describe the following presentations.

A 76-year-old gentleman is having difficulty in recognising complex pictures but can describe the parts. Choose...

An 83-year-old woman having difficulty in copying a wire cube on extended cognitive examination. Choose...

A 73-year-old gentleman with a history of a major stroke suffered from left sided hemiplegia. However, he denies having any physical problems including his gait disturbances. Choose...

A 78-year-old gentleman with Alzheimer dementia has difficulty in recognizing the faces of his children, grandchildren and close friends, However, he recognizes their voices. Choose...

A 76-year-old woman complains of a feeling that the limbs on one side of the body are missing. Choose...

Choose...
Anosognosia
Simultanagnosia
Visuospatial Agnosia
Dressing apraxia
Agraphaesthesia
Ideomotor apraxia
Prosopagnosia
Hemisomatognosia
Astereognosia
Finger Agnosia

Explanation:

Simultanagnosia is impairment in the perception or integration of visual stimuli appearing simultaneously.

Visuospatial agnosia can encompass several impairments, including the impaired ability to copy simple drawings.

Anosognosia is the inability to recognise a physical deficit in oneself (e.g. patient denies paralysed limb).

Prosopagnosia is the inability to recognise familiar faces that is not due to impaired visual acuity or level of consciousness.

Hemisomatognosia is a unilateral lack of body image in which the person behaves as if one side of the body is missing. It can occur in migraine or during an epileptic aura.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 1136, 1143-1144.

Colman AM. Oxford Dictionary of Psychology, 4th Edition. 2015. p. 19.

Casey P, Kelly B. Fish's clinical pathology: Signs and symptoms in psychiatry, 4th Edition. 2019.p. 39.

The correct answer is: A 76-year-old gentleman is having difficulty in recognising complex pictures but can describe the parts. → Simultanagnosia, An 83-year-old woman having difficulty in copying a wire cube on extended cognitive examination. → Visuospatial Agnosia, A 73-year-old gentleman with a history of a major stroke suffered from left sided hemiplegia. However, he denies having any physical problems including his gait disturbances. → Anosognosia, A 78-year-old gentleman with Alzheimer dementia has difficulty in recognizing the faces of his children, grandchildren and close friends, However, he recognizes their voices. → Prosopagnosia, A 76-year-old woman complains of a feeling that the limbs on one side of the body are missing. → Hemisomatognosia

Question **17**

Not answered

Marked out of 6.00

Localising signs

Choose two likely symptoms for each of the following conditions:

A malnourished alcoholic. Choose...

A right-handed person with right parietal lobe lesion. Choose...

A frontal meningioma. Choose...

Choose...
Anosognosia and constructional apraxia
Agraphia and anosognosia
Anosmia and anosognosia
Ataxia and constructional apraxia
Ipsilateral optic atrophy and anosmia
Ataxia and 4th nerve palsy
Ataxia and 6th nerve palsy
Ipsilateral optic atrophy and nystagmus

This patient is most likely to have Wernicke's encephalopathy. The classic triad includes ophthalmoplegia, ataxia and confusion. The most common ophthalmoplegia here is a bilateral 6th nerve (not 4th nerve) lesion. The 4th nerve may be affected in few cases. The non-dominant parietal lobe has a significant role in constructional praxis and also in awareness of illness states. Frontal meningiomas are associated with anosmia (often present due to the common olfactory groove distribution of tumours causing pressure) and ipsilateral optic atrophy, on the side of the tumour. Anosmia with ipsilateral optic atrophy and contralateral papilledema is also known as Kennedy-Foster syndrome.

The correct answer is: A malnourished alcoholic. → Ataxia and 6th nerve palsy, A right-handed person with right parietal lobe lesion. → Anosognosia and constructional apraxia, A frontal meningioma. → Ipsilateral optic atrophy and anosmia

Question **18**

Not answered

Marked out of 5.00

Rating scales in old age psychiatry

Identify the best fitting rating scales that could be used in each of the following situations.

This scale is more useful for the elderly as it focuses on cognitive and subjective symptoms more than physical symptoms. Choose...

This scale is helpful to detect depression in people with dementia. Choose...

This scale is particularly useful for older hearing impaired subjects. Choose...

This scale is helpful for screening people with dementia. Choose...

This scale has a number of somatic items, which renders it less appropriate for older subjects. Choose...

Choose...
Cornell scale for depression
BASDEC
MADRS
Geriatric depression scale
MMSE
CAMDEX
IQ-CODE
Hamilton Depression Rating Scale
AMTS

Explanation:

The Geriatric Depression Scale (GDS) is a 30-item self-report screen for symptoms of depression. Shorter versions of the GDS have been developed, most commonly used is the 15-item version. The yes-no format is less cognitively demanding than other scales. It is a useful screening instrument that excludes somatic complaints from its lists of items.

The Cornell Scale of Depression in Dementia is specifically designed to screen for depression in a population of demented elderly. It is an interviewer-administered scale relying on information from caregivers and clinicians' observations. This scale includes items concerning physical well-being, sleep, appetite, and other vegetative symptoms.

The Brief Assessment Schedule Depression Cards (BASDEC) was developed for use with older people in a hospital ward environment. It is a set of 19 statements relating to depression symptoms, each written on a separate card. The respondent is asked to place the statement card next to either a 'True' or 'False' card according to their current view. The maximum possible score is 21, higher scores indicating greater depressive symptomatology.

The MMSE is a 30-point cognitive test developed in the mid-1970s to provide a bedside assessment of a broad array of cognitive functions, including orientation, attention, memory, construction, and language.

The Hamilton Depression Rating Scale is a widely used scale for measuring depression, consisting of a list of 17 items, designed to be rated by a clinician on three-point and five-point scales. The symptoms are: depressed mood, guilt feelings, suicide, insomnia (initial or early), insomnia (middle), insomnia (terminal or late), disturbed work and activities, psychomotor retardation, psychomotor agitation, anxiety (psychological), anxiety (somatic), gastrointestinal symptoms, somatic symptoms, sexual dysfunction or menstrual disturbance, hypochondriasis, weight loss, and lack of insight.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 45, 54, 90.

Kørner A et al. The geriatric depression scale and the Cornell scale for depression in dementia. A validity study. Nordic journal of psychiatry. 2006 Jan 1;60(5):360-4.

Healey AK et al. A preliminary investigation of the reliability and validity of the Brief Assessment Schedule Depression Cards and the Beck Depression Inventory-Fast Screen to screen for depression in older stroke survivors. International journal of geriatric psychiatry. 2008 May;23(5):531-6.

Colman AM. Oxford Dictionary of Psychology, 4 th Edition. 2015. p. 330.

The correct answer is: This scale is more useful for the elderly as it focuses on cognitive and subjective symptoms more than physical symptoms. → Geriatric depression scale, This scale is helpful to detect depression in people with dementia. → Cornell scale for depression, This scale is particularly useful for older hearing impaired subjects. → BASDEC, This scale is helpful for screening people with dementia. → MMSE, This scale has a number of somatic items, which renders it less appropriate for older subjects. → Hamilton Depression Rating Scale

Question **19**

Not answered

Marked out of 5.00

Biochemical Investigations

Which one of the listed investigations is associated with the assessment of each of the psychiatric conditions given below?

Choose...
Dexamethasone suppression test
Lactate provocation test
Mean corpuscular volume
Narcoanalysis
Urinalysis
Polysomnography
Catecholamine levels
ECG
Serum creatinine phosphokinase
Serum prolactin levels

Panic disorder	Choose...
To differentiate pseudo seizures from true epileptic attacks	Choose...
Neuroleptic malignant syndrome	Choose...
Syndrome of inappropriate ADH secretion induced by antidepressants	Choose...
Depression	Choose...

Lactate infusion: Nearly 72% patients with panic disorder have a panic attack when administered IV injections of sodium lactate. Therefore, lactate provocation is used to confirm a diagnosis of panic disorder. Hyperventilation and CO2 inhalation have been used.

Serum prolactin levels are usually elevated within 15-20 minutes of true seizures, which is not the case

in pseudo seizures.

Creatinine phosphokinase (CPK) is raised in neuroleptic malignant syndrome.

Urine analysis may be necessary to differentiate SIADH induced by antidepressants or antipsychotics from psychogenic polydipsia where excessive water consumption occurs without apparent organic illness or Diabetes inspidus due to lithium(nephrogenic) or head injury (central). The urine osmolality is high in SIADH induced by antidepressants despite a hyponatremia.

Continuous production of endogenous cortisol despite administration of exogenous steroid (dexamethasone) suggests a failure of feedback suppression of ACTH/CRH. Normally administering dexamethasone must reduce cortisol in plasma. This is because of intact HPA function leading to reduced ACTH and CRH. In depression and other psychiatric hypercortisolemic states (also in organic hypercortisolemic states such as Cushing's), this does not occur. The dexamethasone suppression test (DST) is used to demonstrate this hypercortisolemia (feedback failure).

The correct answer is: Panic disorder → Lactate provocation test, To differentiate pseudo seizures from true epileptic attacks → Serum prolactin levels, Neuroleptic malignant syndrome → Serum creatinine phosphokinase, Syndrome of inappropriate ADH secretion induced by antidepressants → Urinalysis, Depression → Dexamethasone suppression test

Question **20**

Not answered

Marked out of 3.00

Jungian concepts in dynamic psychopathology

For each description given below, select the most appropriate Jungian concept from the list provided:

The unconscious masculine aspects of a woman. Choose...

Unacknowledged aspects of oneself that includes both creative impulses and destructive urges. Choose...

The outer concealed aspects of oneself Choose...

Choose...
Anima
Shadow
Animus
Archetypes
Collective unconscious
Self
Persona
Analytical Psychology
Personal unconscious
Introversion
Extroversion

Jung founded analytic psychology and introduced the terms extraversion and introversion. He described collective unconscious - a term describing all of the mankind's collective symbolic past, and archetypes - a term describing images and symbols that constitute the collective unconscious. These representational images have universal symbolic meaning (e.g. Hero, Old Wise Man, Tree, etc.). Other terms described by Jung include Persona which is the mask covering one's personality and presented to outside world, Anima - unconscious feminine aspect of a man, Animus - unconscious masculine character of a woman, Shadow - an archetype - a personification of unacceptable aspects of oneself symbolized as a dark internal alien and Individuation - ultimate goal of life where an individual develops a sense of self- identity.

The correct answer is: The unconscious masculine aspects of a woman. → Animus, Unacknowledged aspects of oneself that includes both creative impulses and destructive urges. → Shadow, The outer concealed aspects of oneself → Persona

Question **21**

Not answered

Marked out of 5.00

Flag question

Eye signs

Given the clinical situation, identify the eye signs commonly seen:

Sluggish light and accommodation reflexes with persistent reduction in pupil size Choose...

Small and irregular pupil that does not react to light but does accommodate Choose...

Choose...
Argyll-Robertson pupil
Holmes- Adie pupil
Dolls eye manoeuvre
Senile pupil
Anisocoria
Marcus Gunn pupil
Kayser-Fleischer ring
Hutchinson pupil
Anosognosia
Horner's syndrome

As the flashlight swings from the normal eye to the abnormal eye, the abnormal pupil dilates. No anisocoria is seen. Choose...

The pupil on one side constricts and then widely dilates. Then the pupil of the other eye goes through the same sequence Choose...

Ptosis, anhidrosis and enophthalmos with normal light and accommodation reflex Choose...

Senile pupil: Note that with aging both accommodation and light reflexes become sluggish. This is accompanied by a persistent reduction in pupil size, called senile miosis. Senile miosis is better observed when the eye is completely dark-adapted i.e. in darkness. The senile pupil is the result of age-related atrophy of the radiator dilator muscle of the pupil despite relatively preserved sphincter constrictor muscle strength.

Argyll-Robertson pupil (ARP), seen in neurosyphilis, is irregular and small; it does not react to light, but does accommodate. (Mnemonic: A-R-P: Accommodation Reflex Present).

Marcus-Gunn pupil: This sign is elicited using a swinging flashlight test. As the flashlight swings from one eye to the other, the abnormal pupil dilates when the light turns away from the normal eye, onto the affected side. At rest, anisocoria (unequal pupils) is not seen in this condition. This phenomenon indicates an afferent pupillary defect arising from optic nerve damage. It also goes by the name paradoxical pupillary reflex.

Hutchinson pupil results from rapidly rising unilateral intracranial pressure, as in sudden massive intracranial haemorrhage.

Horner's syndrome refers to ptosis, anhydrosis and enophthalmos with normal light and accommodation reflex.

Ref: Neurological History and Physical Examination. (n.d.). Retrieved from http://emedicine.medscape.com/article/1147993-overview

The correct answer is: Sluggish light and accommodation reflexes with persistent reduction in pupil size → Senile pupil, Small and irregular pupil that does not react to light but does accommodate → Argyll-Robertson pupil, As the flashlight swings from the normal eye to the abnormal eye, the abnormal pupil dilates. No anisocoria is seen. → Marcus Gunn pupil, The pupil on one side constricts and then widely dilates. Then the pupil of the other eye goes through the same sequence → Hutchinson pupil, Ptosis, anhidrosis and enophthalmos with normal light and accommodation reflex → Horner's syndrome

Question **22**

Not answered

Marked out of 4.00

Flag

Gait abnormality

Choose the best option for each of the following;

Huntington disease Choose...

Pregnancy Choose...

Hip dysplasia Choose...

Friedreich ataxia Choose...

Choose...
Stomping gait
Failure of gait ignition
Myopathic gait (Waddling gait)
High stepping gait (Steppage gait)
Broad-based, unsteady gait
Stiff legged, scissoring gait
Festinant gait (shuffling gait)
Lurching chaotic gait
Pigeon gait

A chaotic lurching gait is seen in Huntington disease.

Waddling gait is a broad-based gait with a duck-like waddle to the swing phase seen in pregnancy and

proximal myopathy.

Torsional difficulties lead to pigeon gait in hip dysplasia.

In dorsal column sensory disturbances such as subacute combined cord degeneration and Friedreich ataxia, a stomping gait is noted.

The correct answer is: Huntington disease → Lurching chaotic gait, Pregnancy → Myopathic gait (Waddling gait), Hip dysplasia → Pigeon gait, Friedreich ataxia → Stomping gait

Question **23**

Not answered

Marked out of 4.00

Choose...
SANS
SCAN
CATEGO
HONOS
PANSS
HAMD
BPRS
HADS
CAN

Validated rating scales for clinical use

Choose one rating scale for each of the following;

A primary care research student would like to use a computer programme to generate psychiatric diagnosis. Choose...

A CT1 trainee would like to use a semi-structured interview, which retains the features of a clinical examination . Choose...

A rehabilitation nurse would like to carry out a needs assessment looking at various domains of an individual's life. Choose...

The commissioners of the local catchment area would like mental health professionals to use an instrument, which could measure clinical outcomes in mental health. Choose...

Explanation:

CATEGO is computer software for processing SCAN data according to categories of signs and symptoms.

The Schedules for Clinical Assessment in Neuropsychiatry (SCAN) has evolved from the older Present State Examination (PSE). SCAN is a set of instruments aimed at assessing and classifying psychopathology in adults. Instruments include the PSE-10 (the tenth edition of the Present State Examination), the SCAN glossary, the Item Group Checklist, and the Clinical History Schedule. The interview is semi-structured, the aim being to encapsulate the clinical interview while minimising its vagaries. Where there is doubt, the interviewer can proceed to a freestyle interview to clarify further. It is designed for use by psychiatrists or clinical psychologists, thereby utilising clinical interviewing skills in evaluating each symptom. The use of mental health professionals in interviewing with SCAN makes this an expensive method but has the advantage of approximating the 'gold standard' diagnosis achieved by clinical interview.

The Camberwell Assessment of Need (CAN) is an interviewer-administered instrument for comprehensive assessment of the health and social needs of patients with severe and enduring mental illness, principally disorders within the schizophrenia spectrum. Originally developed in the early 1990s for needs assessment associating the British National Health Service (NHS) and Community Care Act 1990, it has since been disseminated internationally.

The Health of the Nation Outcomes Scales (HoNOS) is a 12-scale clinician-rated measure developed by the Royal College of Psychiatrists to guide everyday clinical practice and measure health and social care outcomes in secondary mental health service for adults age 18-65 years. It was designed to be sensitive to improvement, deterioration, or lack of change over time.

Ref: Colman AM. Oxford Dictionary of Psychology, 4 th Edition. 2015. p. 115.

Casey P, Kelly B. Fish's clinical pathology: Signs and symptoms in psychiatry, 4 th Edition. 2019. p.

Wennström E. The Camberwell assessment of need as an outcome measure in community mental health care. Acta Universitatis Upsaliensis 2008;310.

James M et al. A review and update of the Health of the Nation Outcome Scales (HoNOS). BJPsych bulletin. 2018 Apr;42(2):63-8.

The correct answer is: A primary care research student would like to use a computer programme to generate psychiatric diagnosis. → CATEGO, A CT1 trainee would like to use a semi-structured interview, which retains the features of a clinical examination. → SCAN, A rehabilitation nurse would like to carry out a needs assessment looking at various domains of an individual's life. → CAN, The commissioners of the local catchment area would like mental health professionals to use an instrument, which could measure clinical outcomes in mental health. → HONOS

Question **24**

Not answered

Marked out of 12.00

Epilepsy & clinical features

According to the most important features seen in various types of epilepsy, identify the clinical syndrome described.

It may start with a focal lesion in the brain and may later have a secondary generalization — Choose...

Seizures differ from attack to attack with features such as arching of the back and asymmetrical jerking of limbs. Intensity of seizure influenced by the presence of an observer — Choose...

Loss of awareness is an important feature. EEG shows multispikes followed by spike and slow waves — Choose...

It may start with a focal lesion in the brain with secondary generalization. Loss of awareness is an important feature. — Choose...

EEG shows generalised 3 Hz spike and slow wave activity. Loss of awareness is an important feature — Choose...

Choose...
Absence seizures
Simple partial seizures
Infantile spasms
Grandmal seizures
Dissociative seizures
Myoclonic seizures
Complex partial seizures

A simple partial seizure arises in a localized area of the cerebral cortex, and it evolves into a secondarily generalized seizure.

Dissociative seizures are non-epileptic events that have a psychogenic origin and resemble epileptic seizures. Here the patients can present with bizarre and irregular movements such as arching of the limbs and asymmetrical jerking of the limbs. The behaviour is also influenced by the presence of an observer, by restraint or by suggestion during periods of apparent unresponsiveness.

In grandmal seizures, loss of awareness is a key feature and EEG shows multispikes followed by spike and slow waves.

Complex partial seizures often start as simple partial seizures and progress to impairment of consciousness.

Absence seizures occur without warning and consist of a sudden interruption of consciousness. The patient abruptly ceases whatever activity he or she is engaged in, and the eyes stare vacantly ahead. It usually lasts 2-15 seconds following which normal behaviour resumes and the patient is often unaware of what has happened. Typical absence seizures are accompanied by a characteristic EEG abnormality of generalized spike and wave activity at a frequency of 3 Hz (2-4 Hz spike and wave activity)

The correct answer is: It may start with a focal lesion in the brain and may later have a secondary generalization → Simple partial seizures, Seizures differ from attack to attack with features such as arching of the back and asymmetrical jerking of limbs. Intensity of seizure influenced by the presence of an observer → Dissociative seizures, Loss of awareness is an important feature. EEG shows multispikes followed by spike and slow waves → Grandmal seizures, It may start with a focal lesion in the brain with secondary generalization. Loss of awareness is an important feature. → Complex partial seizures, EEG shows generalised 3 Hz spike and slow wave activity. Loss of awareness is an important feature → Absence seizures

Question **25**

Not answered

Marked out of 3.00

Speech difficulties in children

Identify the type of speech difficulties seen in each of the following situations:

A 7-year-old girl can't speak but can utter incomprehensible sounds. She can make herself understood using non-verbal communication and can read and understand. — Choose...

A 7-year-old boy says, "Me want a drink" rather than "I would like a drink." — Choose...

An 11-year-old boy says, "In deep space nine soldiers are fronting" when asked if he wants some food. He appears to stare vacantly in response to other questions. — Choose...

Choose...
Autism
Speech fluency deficit
Expressive language disorder
Dysphonia
Dysarthria
Dyslexia
Elective mutism
Schizophrenia

In expressive language disorder, the use of non-verbal cues and internal language, as reflected in imaginative or make-believe play, are intact, and the capacity to interact socially without words is almost preserved. The child shows interest in communicating despite its limitations and uses demonstration, gesture or non-speech vocalization to compensate for lack of verbal language (ICD-10; 238).

Autism- Speech is not only limited in autism but is pervaded by echolalia, stereotypic utterances and avoidance of the personal pronoun like I, etc.

Case3: This boy appears to have childhood-onset schizophrenia, with thought disorder characterised by "breaks or interpolations in the train of thought, resulting in incoherence or irrelevant speech or neologisms." (ICD-10)
Schizophrenia and Public Health. (n.d.). Retrieved from http://www.who.int/mental_health/media/en/55.pdf

The correct answer is: A 7-year-old girl can't speak but can utter incomprehensible sounds. She can make herself understood using non-verbal communication and can read and understand. → Expressive language disorder, A 7-year-old boy says, "Me want a drink" rather than "I would like a drink." → Autism, An 11-year-old boy says, "In deep space nine soldiers are fronting" when asked if he wants some food. He appears to stare vacantly in response to other questions. → Schizophrenia

Question **26**

Not answered

Marked out of 3.00

Psychiatric syndromes associated with stroke

For each of the following clinical conditions below, match the associated psychiatric syndrome from the given list:

After an acute haemorrhagic stroke, a 76-year-old woman becomes anxious and tearful whenever she attempts to solve times crossword. — Choose...

An 80-year-old gentleman with right-sided hemiparesis presents with impairment in the control of crying. — Choose...

A 67-year-old man is recovering after a significant thromboembolic stroke. He appears unconcerned about his memory loss and not motivated to participate in the physical rehabilitation activities. — Choose...

Choose...
Mania
Emotionalism
Catastrophic Reaction
Psychosis
Emotional lability
Cognitive impairment
Apathy without depression
Depression
Anxiety disorder
Bipolar affective disorder

A catastrophic reaction is seen in dementia when a cognitively impaired patient is asked undertake tasks beyond their performance ability. Patients may appear agitated, angry and even violent. The situation described as case 2 is termed emotionalism - a recurrent crying (or sobbing) or laughing episodes that mark a change in previous behaviour. Post-stroke emotionalism has a high prevalence rate varying between 15% to 35%. Apathy is dull emotional tone associated with detachment, which mimics like depression. Often there is avolition and reduced motivation. No other overt symptoms of depression are present. This is seen in 20% of patients with stroke.

The correct answer is: After an acute haemorrhagic stroke, a 76-year-old woman becomes anxious and tearful whenever she attempts to solve times crossword. → Catastrophic Reaction, An 80-year-old gentleman with right-sided hemiparesis presents with impairment in the control of crying. → Emotionalism, A 67-year-old man is recovering after a significant thromboembolic stroke. He appears unconcerned about his memory loss and not motivated to participate in the physical rehabilitation activities. → Apathy without depression

Question **27**

Not answered

Marked out of 4.00

Somatoform and Dissociative Disorders

For each of the following descriptions choose the most appropriate diagnosis from the list provided:

A 47-year-old woman presented with a 4-year history of multiple vague physical symptoms not explained by any physical causes. She seeks repeated consultations with her GP and hospital specialists. — Choose...

A 35-year-old gentleman who presents himself 30 miles away from his residence for no apparent reason with well-maintained self-care. — Choose...

A 43-year-old prisoner awaiting trial when asked how many legs a cat has, answers 'five legs.' — Choose...

A 40-year-old woman who is always preoccupied that she has breast cancer despite all her relevant investigations were proved to be negative. — Choose...

Choose...
Hypochondriacal disorder
Dissociative amnesia
Neurasthenia
Ganser's syndrome
Dissociative fugue
Possession disorder
Munchausen syndrome
Somatisation disorder
Multiple personality disorder

Somatisation is the experience of bodily symptoms with no, or no sufficient, physical cause for them, with presumed psychological causation.

Fugue is a dissociative reaction to intolerable stress. This could happen following a significant external stressor that the affected individual develops global amnesia and may wander to a distant location. Consciousness is unimpaired.

Ganser syndrome involves the production of approximate answers. Here the patient gives repeated wrong answers to questions, which are nonetheless in the right ballpark. Sometimes this is associated with organic brain illness though it is much more common as a form of malingering (especially in prisoners anticipating trial).

In hypochondriasis, patient looks for diagnosis, not symptom relief; s/he often names a particular suspect physical disorder and may be more or less convinced about having the disorder. In a patient with somatisation musculoskeletal symptoms are common; while in hypochondriasis gastrointestinal problems are the most common ones. Family history of alcohol use and antisocial personality may be more common in women with somatisation disorder

The correct answer is: A 47-year-old woman presented with a 4-year history of multiple vague physical symptoms not explained by any physical causes. She seeks repeated consultations with her GP and hospital specialists. → Somatisation disorder, A 35-year-old gentleman who presents himself 30 miles away from his residence for no apparent reason with well-maintained self-care. → Dissociative fugue, A 43-year-old prisoner awaiting trial when asked how many legs a cat has, answers

'five legs.' → Ganser's syndrome, A 40-year-old woman who is always preoccupied that she has breast cancer despite all her relevant investigations were proved to be negative. → Hypochondriacal disorder

Question **28**

Not answered

Marked out of 3.00

Tropical diseases

Choose the possible infective agent for each of the following presentations.

An 18-year-old girl develops a circular bull's eye rash with lethargy and myalgia along with memory and mood disturbances after returning from a camping trip in America. — Choose...

A 36-year-old man running an animal and bird rescue centre presents with high fever, joint pains along with diarrhoea. On examination, he has conjunctivitis and macular spots on his face. — Choose...

A 65-year-old Vietnam war veteran presents with manic features and cognitive impairment. He has poor balance and complains of lightning pains in the lower extremities when examined. — Choose...

Choose...
Leptospirosis
Epstein-Barr Virus
Treponema pallidum
Giardia lamblia
Borrelia burgdorferi
Cytomegalovirus
Leishmania Donavanii
Tropical yaws
Falciparum malaria
Chlamydia psittaci
Trichomonas

Explanation:

Lyme disease is caused by infection with the spirochete Borrelia burgdorferi transmitted through the bite of the deer tick, which feeds on infected deer and mice. A characteristic bull's eye rash occurs at the site of the tick bite followed shortly after that by flulike symptoms. Impaired cognitive functioning and mood changes are associated with the illness and may be the presenting complaint. These include memory lapses, difficulty concentrating, irritability, and depression.

Chlamydia psittaci causes psittacosis in humans, mainly in persons in contact with birds in either the setting of occupational or companion bird exposure. Psittacosis can result in an asymptomatic infection or an acute symptomatic infection characterised by mild flu-like illness to severe atypical pneumonia and systemic disease. Persons suffering from a symptomatic infection can have headache, chills, fever (up to 40.5 degrees Celsius), malaise, and myalgia. Other symptoms are rare but keratoconjunctivitis, gastrointestinal problems (vomiting, abdominal pain, and diarrhoea), arthritis, icterus, myocarditis, endocarditis, myeloradiculitis, encephalitis, meningoencephalopathy, optic neuritis, and multiorgan failure have been reported.

Neurosyphilis (also known as general paresis) appears 10-15 years after the primary Treponema pallidum infection. Neurosyphilis generally affects the frontal lobes and results in personality changes, the development of poor judgment, irritability, and decreased care for self. Delusions of grandeur develop in 10-20% of affected patients.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 255-256.

Rybarczyk et al. Human psittacosis: a review with emphasis on surveillance in Belgium. Acta Clinica Belgica. 2019 Mar 18.

The correct answer is: An 18-year-old girl develops a circular bull's eye rash with lethargy and myalgia along with memory and mood disturbances after returning from a camping trip in America. → Borrelia burgdorferi, A 36-year-old man running an animal and bird rescue centre presents with high fever, joint pains along with diarrhoea. On examination, he has conjunctivitis and macular spots on his face. → Chlamydia psittaci, A 65-year-old Vietnam war veteran presents with manic features and cognitive impairment. He has poor balance and complains of lightning pains in the lower extremities when examined. → Treponema pallidum

Question **29**

Not answered

Marked out of 5.00

Choose...
Vertical hemianopia
Horizontal hemianopia
Funnel vision
Quadrantanopia
Tunnel vision
Argyll Robertson pupil
Adie pupil
Anton's syndrome
Cortical blindness

Visual defects

Using the descriptions given below, identify the type of visual defect

A 79-year-old man with a recent history of major stroke describes having a keyhole vision — Choose...

A 45-year-old woman with sudden onset blindness describes patchy spirals of field loss — Choose...

A 67-year-old man has a history of glaucoma. He reports that the field projected at 2 meters is larger than the one plotted at 1 metre. — Choose...

A 60-year-old woman who is blind denies her visual defect — Choose...

A 70-year-old woman with optic nerve lesions describes only one-fourth of the field is affected on one side — Choose...

Cortical blindness is most often due to posterior cerebral artery occlusion. In effect, it is a bilateral homonymous hemianopia. Patients experience either a small central field around the point of fixation (macular sparing or keyhole vision) or complete blindness.

For case 2, the diagnosis is conversion disorder. Tunnel vision refers to the absence of projection distance disparity in the field of vision and reporting patchy spirals of field loss. Tunnel vision is seen in hysteria or malingering.

Funnel vision: In the presence of an organic field defect, the field projected at two metres is larger than the field plotted at one metre (funnel vision). This is seen in glaucoma, retinitis pigmentosa, in the presence of hyaline bodies in the disc, post papilledema optic atrophy, bilateral occipital infarcts with macular sparing, and feigned visual loss.

Occasionally, patients with cortical blindness deny their visual defect, which is called Anton's syndrome.

When only a one-fourth of the field is affected, the resulting deficit is called quadrantanopia. Hemianopia is a field defect that encompasses roughly half of the field. Vertical hemianopia can be nasal or temporal. Horizontal or altitudinal hemianopia can be superior or inferior. Bilateral field defects are said to be homonymous when they are similarly located in both visual fields.

The correct answer is: A 79-year-old man with a recent history of major stroke describes having a keyhole vision → Cortical blindness, A 45-year-old woman with sudden onset blindness describes patchy spirals of field loss → Tunnel vision, A 67-year-old man has a history of glaucoma. He reports that the field projected at 2 meters is larger than the one plotted at 1 metre. → Funnel vision, A 60-year-old woman who is blind denies her visual defect → Anton's syndrome, A 70-year-old woman with optic nerve lesions describes only one-fourth of the field is affected on one side → Quadrantanopia

Question **30**

Not answered

Marked out of 2.00

Choose...
CT scan
MRI scan
ECG
F-MRI scan
MRS scan
Lumbar puncture
Ultrasound scan
SPECT scan
EEG

Investigations of choice

Choose the most appropriate investigations of choice for following clinical situations.

A 35-year-old gentleman presenting with fever, abrupt onset confusion and behavioral changes — Choose...

A 75-year-old gentleman, who suffers from late onset psychosis, has been on olanzapine. He develops parkinsonian symptoms, and the neurologist says that it could be antipsychotic-induced. — Choose...

Case 1: This presentation is suggestive of herpes simplex encephalitis (HSE) characterized by a fairly abrupt onset of confusion, memory impairment, and often seizures. 70% show psychiatric disturbances including acute confusion, depression and psychosis. Fever is common but not invariable. Magnetic resonance imaging (MRI) provides the most sensitive method of detecting early lesions and is the imaging of choice in HSE. Cerebrospinal fluid (CSF) examination commonly reveals a lymphocytosis and raised protein. The gold standard for in vivo diagnosis is CSF polymerase chain reaction (PCR) for herpes viruses, which has a sensitivity and specificity of ~95%.

Case 2: This could be a case of Lewy body dementia presenting initially with psychotic symptoms and later developed other features like motor symptoms of parkinsonism and neuroleptic sensitivity. Alternatively the patient might be now developing Parkinson's disease on top of functional psychotic illness. In either case, the investigation useful would be DAT-SCAN (I-123-FP-CITscan). This uses SPECT technology and can be undertaken in most imaging departments. This demonstrated abnormal images in 86% of DLB cases and also helps to differentiate idiopathic Parkinson's from drug-induced parkinsonism.

The correct answer is: A 35-year-old gentleman presenting with fever, abrupt onset confusion and behavioral changes → MRI scan, A 75-year-old gentleman, who suffers from late onset psychosis, has been on olanzapine. He develops parkinsonian symptoms, and the neurologist says that it could be antipsychotic-induced. → SPECT scan

Question **31**

Not answered

Marked out of 5.00

Flag

DSM Axes

Identify the descriptions from the given list that are best suited for each of the following DSM-IV axes.

Axis-2 Choose...

Axis-3 Choose...

Axis-4 Choose...

Choose...
Complex partial seizures
Recurrent self-harm
Moderate mental retardation
Global assessment of functioning=60
Suicidality
Alcohol intoxication
Anorexia nervosa
Narcolepsy
Unemployment
Psychotic disorder due to a general medical condition

The multi-axial system of DSM-IV used five axes. Though these axes are now not employed in DSM-5, the multiaxial concept is still being used by researchers and health service analysts.

Axis I - Clinical Disorders (e.g. anorexia, alcohol intoxication, psychosis due to medical condition), Axis II - Personality Disorders/ Mental Retardation (e.g. moderate Mental Retardation, anxious personality traits), Axis III - General Medical Conditions (e.g. epilepsy, hypertension), Axis IV - Psychosocial and Environmental Problems (e.g. Unemployment), Axis V - Global Assessment of Functioning (e.g. GAF score of 60)

The correct answer is: Axis-2 → Moderate mental retardation, Axis-3 → Complex partial seizures, Axis-4 → Unemployment

Question **32**

Not answered

Marked out of 2.00

Flag question

Cognitive impairment in the elderly

For each of the following descriptions choose the most appropriate diagnosis from the list provided.

A 67-year-old gentleman presenting with memory problems and urinary incontinence. On neurological examination, he has a broad-based, short stepped gait. A CT brain scan shows ventricular enlargement. Choose...

Choose...
Sporadic CJD
Depressive pseudodementia
Dementia in Pick's disease
Normal pressure hydrocephalus
Multi-infarct dementia
Dementia in HIV diseases
Dementia in Huntington's disease
Alzheimer's disease
Wilson's' disease.
Lewy body Dementia
Charles Bonnet Syndrome

83-year-old lady with bilateral cataracts sees animals and people in the house. She is independent with all her daily activities and enjoys life. Choose...

Explanation:

Normal pressure hydrocephalus is a syndrome where there is dilatation of cerebral ventricles (especially third ventricle), but normal CSF pressure at lumbar puncture. It typically presents with the triad of dementia, gait disorder, and urinary incontinence. Gait is broad-based, bradykinetic, and shuffling. CT scan shows increased size of the lateral ventricles and thinning of the cortex.

Charles Bonnet syndrome (phantom visual images) is a condition in which complex visual hallucinations occur in the absence of any psychopathology and in clear consciousness. It is associated with either central or peripheral reduction in vision. The images may be static or in motion. They may be enjoyable or distressing. Patients may retain insight.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 154.

Casey P, Kelly B. Fish's clinical pathology: Signs and symptoms in psychiatry, 4th Edition. 2019. pp 29, 124.

The correct answer is: A 67-year-old gentleman presenting with memory problems and urinary incontinence. On neurological examination, he has a broad-based, short stepped gait. A CT brain scan shows ventricular enlargement. → Normal pressure hydrocephalus, 83-year-old lady with bilateral cataracts sees animals and people in the house. She is independent with all her daily activities and enjoys life. → Charles Bonnet Syndrome

Question **33**

Not answered

Marked out of 3.00

Eponymous syndromes in psychiatry

Identify the syndromes in psychiatry based on the descriptions given below

Demyelination and necrosis of corpus callosum associated with red wine consumption. Choose...

Lesion of occipital lobes with cortical blindness. Choose...

Lesion of the dominant parietal lobe. Choose...

Choose...
Klein-Levine syndrome
Geschwind syndrome
Briquet syndrome
Anton syndrome
Gerstmann syndrome
Balint syndrome
Kluver-Bucy syndrome
Marchiafava-Bignami disease

Explanation:

Marchiafava-Bignami disease is a rare disorder, the pathophysiology of which is unknown. It is observed in the setting of chronic alcoholism of long duration and great severity. The macroscopic appearance of the lesions is that of partially necrotic areas of damage in the inner portions of the corpus callosum, with relative preservation of a thin strip of myelinated fibres on its dorsal and ventral surfaces. Histologically, the lesions show loss of myelin with abundant lipid-laden macrophages and relative sparing of axons.

Bilateral damage of the occipital lobes, which contain the visual cortex, can produce severe visual impairment, called cortical blindness. The dramatic neuropsychological phenomenon of Anton syndrome – in which patients explicitly or implicitly deny that they have lost all vision – characteristically complicates the sudden loss of blindness. Some patients demonstrate anosognosia; some simply refuse to admit that they have lost vision; while others blame external factors, like dim light, for their problem.

Gerstmann syndrome is characterised by left-right disorientation, agraphia, acalculia, and finger

agnosia, generally associated with lesions in the parietal lobe of the dominant cerebral hemisphere.

Ref: Gray F, Duyckaerts C, de Girolami U. Escourolle and Poirier's Manual of Basic Neuropathology, 6th Edition. 2019. pp. 231-232.

Kaufman DM, Geyer HL, Milstein MJ, Rosengard JL. Kaufman's Clinical Neurology for Psychiatrists, 9th Edition. 2023. p. 276.

Colman AM. Oxford Dictionary of Psychology, 4th Edition. 2015. p. 313.

The correct answer is: Demyelination and necrosis of corpus callosum associated with red wine consumption. → Marchiafava-Bignami disease, Lesion of occipital lobes with cortical blindness. → Anton syndrome, Lesion of the dominant parietal lobe. → Gerstmann syndrome

Question **34**

Not answered

Marked out of 5.00

Choose...
restatement
summation
symptom expectation
limit setting
redirection
normalisation
clarification
repetition
facilitation
acknowledgement
self-revelation

Communication skills

Identify the interview techniques used in each of the following situations:

Patient says: 'It is not good if one's parents are divorced even before one goes to school.' Doctor says: 'I'd like to hear more about your parents, but first let me get a picture of what's happening to you of late'. — Choose...

When discussing alcohol use, suddenly the clinician asks 'What sorts of drugs do you usually use when you're drinking?' — Choose...

'Sometimes when people are very depressed, they think of hurting themselves. Has this been true for you?' — Choose...

'I can see that you look anxious when talking about those voices'. — Choose...

Clinician says 'In fact when I took fluoxetine some years ago, I had all these side effects for first few days. But they disappeared in few weeks' time.' — Choose...

Explanation:

Redirection is often crucial to a successful interview because of the time constraints and the necessity to obtain a broad overview of the patient's life as well as the current problems. Also, the patient may, for conscious or often unconscious reasons, avoid certain important areas, and need guidance in approaching these subjects. Redirection can be used when the patient changes the topic or when the patient continues to focus on a non-productive or well-covered area.

In symptom expectation, the clinician expects that the patient has behaved in a certain way after listening to some information.

Normalisation is a validity technique for raising a sensitive or taboo topic that makes it easier for the patient to feel comfortable exploring the topic. In normalisation, the interviewer phrases the question so that the patient realizes that they are not the only person who has ever experienced the behaviours or problems under scrutiny. The clinician meta-communicates that other patients describe similar thoughts, feelings, or experiences.

Acknowledgement refers to the clinician openly acknowledging some aspect present in the interview.

Self-revelation (self-disclosure or self-transparency) is an engagement technique that can powerfully backfire. In most instances this is not recommended unless one feels that it is necessary for engagement with a specific patient.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 15.

Shea SC. Psychiatric interviewing: The art of understanding, 3rd Edition. 2016. pp. 158-159, 795.

The correct answer is: Patient says: 'It is not good if one's parents are divorced even before one goes to school.' Doctor says: 'I'd like to hear more about your parents, but first let me get a picture of what's happening to you of late'. → redirection, When discussing alcohol use, suddenly the clinician asks 'What sorts of drugs do you usually use when you're drinking?' → symptom expectation, 'Sometimes when people are very depressed, they think of hurting themselves. Has this been true for you?' → normalisation, 'I can see that you look anxious when talking about those voices'. → acknowledgement, Clinician says 'In fact when I took fluoxetine some years ago, I had all these side effects for first few days. But they disappeared in few weeks' time.' → self-revelation

Question **35**

Not answered

Marked out of 4.00

Clinical neurological signs

Identify the neurological sign seen in each of the following situations:

A 60-year-old man develops respiratory failure, which results in anoxic brain damage. (Select ONE). Choose...

A 35-year-old man is a known binge drinker with significant malnourishment. He present with acute onset confusion, ataxia and disorientation. Choose...

An 18-year-old man sustains motorbike accident with spinal injury at the level of T5. Choose...

Choose...
Perseveration
Intention tremor
Upper limb rigidity
Extensor plantar reflex
Lhermitte's sign
Straight leg rising sign
Beevor sign
Optic atrophy
Flapping tremor
Nystagmus
Neck stiffness

A flapping tremor is seen in cases of respiratory, hepatic or cardiac failure with encephalopathy.Ataxia and ophthalmoplegia with nystagmus in a background of impaired consciousness are characteristic of Wernicke's encephalopathy.Beevor's sign is characterised by an upward movement of umbilicus when raising the head from a supine position due to lower abdominal paralysis.

The correct answer is: A 60-year-old man develops respiratory failure, which results in anoxic brain damage. (Select ONE). → Flapping tremor, A 35-year-old man is a known binge drinker with significant malnourishment. He present with acute onset confusion, ataxia and disorientation. → Nystagmus, An 18-year-old man sustains motorbike accident with spinal injury at the level of T5. → Beevor sign

Question **36**

Not answered

Marked out of 6.00

Alcohol-induced physical symptoms

Choose the symptoms which would best support each diagnosis in an alcoholic, malnourished middle-aged man who becomes confused two days after emergency surgery for a femoral shaft fracture.

Bacterial septicaemia Choose...

Wernicke syndrome Choose...

Acute hepatic failure Choose...

Choose...
Elevated central venous pressure
Clonus
Exophtholmos
Flapping tremor
Kernig's sign
Gait ataxia
Nystagmus
Petechial rash

Please note that this patient has recently had an emergency surgery of the femur. Don't try to do Kernig's test on him; also gait ataxia may not be demonstrable.

Wernicke's syndrome is an acute condition characterized by the well-known triad of ophthalmoplegia, ataxia and global confusional state. The classic triad is only present in 10% of patients, and 80% of patients are not diagnosed before postmortem. The eye signs are often bilateral though need not be

always symmetrical.

Jaundice and flapping tremor (Asterixis) are features of hepatic failure. Asterixis can be elicited by having the patient extend both arms with the wrists dorsiflexed and palms facing forward, and eyes closed. Brief jerky downward movements of the wrist are considered a positive sign. Asterixis is commonly seen with metabolic encephalopathies. (Note pronator drift is elicited by having the patient extend both arms with the wrists supinated and palms facing upwards and eyes closed - slow unequal drift towards pronation indicates hemiparetic weakness). Elevated JVP is not seen in hepatic failure that is due to non-cardiac causes.

The correct answer is: Bacterial septicaemia → Petechial rash, Wernicke syndrome → Nystagmus, Acute hepatic failure → Flapping tremor

Question **37**

Not answered

Marked out of 2.00

Complications of Eating Disorders

Identify the metabolic abnormalities commonly seen in the following clinical situations

A 16-year-old girl was admitted to a psychiatric unit for treatments as her weight was dangerously low. She died suddenly when she was administered parenteral nutrition. — Choose...

A 15-year-old girl with a history of anorexia was admitted to a medical ward for severe dehydration. During fluid resuscitation, she went into a state of coma. — Choose...

Choose...
Hypothyroidism
Hyperkalemia
Low T3 and high growth hormone
Hypercalcaemia
Hypercortisolemia
Hypophosphatemia
Hyponatraemia
Hyperphosphatemia
High T3 and high growth hormone
Hypernatraemia

Explanation:

As noted in multiple sources, including MARSIPAN (Management of Really Sick Patients with Anorexia Nervosa), the potentially fatal refeeding syndrome is caused by the switch from fasting gluconeogenesis to carbohydrate-induced insulin release, inducing considerable intracellular uptake of potassium, phosphate, and magnesium from the bloodstream. This can cause profound biochemical derangements including hypokalaemia, hypophosphataemia, hypomagnesaemia, and occasionally hypocalcaemia.

When intravenous replacement of fluids is required, the patient should be transferred to a general medical ward. Uncontrolled correction of dehydration and electrolyte abnormalities can result in hyponatraemia and cerebral oedema.

Ref: Morris J, Anderson S. An update on eating disorders. BJPsych Advances. 2020:1-11.

Taylor DM, Gaughran F, Pillinger T. The Maudsley Practice Guidelines for Physical Health Conditions in Psychiatry. 2021. p. 277.

The correct answer is: A 16-year-old girl was admitted to a psychiatric unit for treatments as her weight was dangerously low. She died suddenly when she was administered parenteral nutrition. → Hypophosphatemia, A 15-year-old girl with a history of anorexia was admitted to a medical ward for severe dehydration. During fluid resuscitation, she went into a state of coma. → Hyponatraemia

Question **38**

Not answered

Marked out of 4.00

Flag question

Bipolar Affective disorder (BPAD)

Select one diagnostic option for each of the following scenarios

A patient, who has been diagnosed with depression and was started on venlafaxine, now presents a history of feeling high with 'unstoppable energy'. — Choose...

Choose...
Treatment-emergent affective switch
Rapid cycling bipolar disorder.
Affective instability
Cyclothymic disorder
Bipolar type 2
Mixed affective state
Dysthymia
Bipolar type 1
Hypomania

A patient who is known to have mood disturbances is taking lithium. In last six months, he has had one episode each of low mood and mania, followed by another episode of high energy, sleep loss and euphoric mood. After an intervening period of normal mood, he developed another episode of mania.

Choose...

A 33-year-old man presents with a history of numerous periods with hypomanic symptoms and numerous periods with depressive symptoms in last two years that do not meet criteria for a mania or a major depressive episode.

Choose...

A 25-year-old woman has episodes of depressed mood and irritability. She described her energy levels to be high and is sleeping only for 4 hours a day.

Choose...

Explanation:

Switching from depression to mania/hypomania in the setting of antidepressant treatment is known as treatment-emergent affective switch (TEAS)

'Rapid cycling' refers to the occurrence of four or more mood episodes per year.

As defined by ICD-11, cyclothymic disorder is characterised by a persistent instability of mood over a period of at least 2 years, involving numerous periods of hypomanic and depressive symptoms that are present more of the time than not. The hypomanic symptomatology may or may not be sufficiently severe or prolonged to meet the full definitional requirements of a hypomanic episode, but there is no history of manic or mixed episodes. The depressive symptomatology has never been sufficiently severe or prolonged to meet the diagnostic requirements of a depressive episode.

A mixed affective state is characterised by the occurrence of both manic/hypomanic and depressive symptoms in a single episode. In ICD-11, a mixed episode is characterised by the presence of several prominent manic and several prominent depressive symptoms consistent with those observed in manic episodes and depressive episodes, which either occur simultaneously or alternate very rapidly (from day to day or within the same day). In DSM-5, the specifier 'with mixed features' can be added to a manic, hypomanic, or depressive episode.

Ref: Gitlin MJ. Antidepressants in bipolar depression: an enduring controversy. International journal of bipolar disorders. 2018 Dec;6(1):1-7.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 328.

Ref: International Statistical Classification of Diseases and Related Health Problems (**11th** ed,; **ICD-11**; World Health Organization, 2020)

The correct answer is: A patient, who has been diagnosed with depression and was started on venlafaxine, now presents a history of feeling high with 'unstoppable energy'. → Treatment-emergent affective switch, A patient who is known to have mood disturbances is taking lithium. In last six months, he has had one episode each of low mood and mania, followed by another episode of high energy, sleep loss and euphoric mood. After an intervening period of normal mood, he developed another episode of mania. → Rapid cycling bipolar disorder., A 33-year-old man presents with a history of numerous periods with hypomanic symptoms and numerous periods with depressive symptoms in last two years that do not meet criteria for a mania or a major depressive episode. → Cyclothymic disorder, A 25-year-old woman has episodes of depressed mood and irritability. She described her energy levels to be high and is sleeping only for 4 hours a day. → Mixed affective state

Question **39**

Diagnosis of anxiety disorders

Not answered

Marked out of 3.00

Identify the most likely diagnosis using the clinical descriptions given below.

A patient with long-standing anxiety describes living on the edge all his life. He has recently deteriorated. He is performing poorly at work. — Choose...

A 42-year-old mother with no previous psychiatric history presents with confusion and highly anxious. Her son developed an acute neurological illness and had been hospitalised since the previous day. — Choose...

A 45 year-old-woman is noted to suffer from anxiety, depression, poor concentration, irritability and is unable to work for last four weeks. She divorced her husband recently. — Choose...

Choose...
Adjustment disorder
Narcissistic personality disorder
Post-traumatic stress disorder
Schizophrenia
Acute stress reaction
Obsessive compulsive disorder
Anankastic personality disorder
Bipolar Affective Disorder
Generalised Anxiety Disorder

Generalised anxiety disorder (case 1) is characterised by prominent tension, excessive worry with generalised free-floating persistent anxiety ('living on the edge') and feelings of apprehension about everyday events leading to significant stress and functional impairment.

Acute stress reaction (case 2) usually starts within an hour of the exposure to the stressful event; resolution starts within 8 hours if the stress is temporary or after 48 hours if it is prolonged. The presence of physical exhaustion, organic factors or physical disease states increases the risk. The stressor is usually one that poses a serious threat to security, integrity and social position. It has a polymorphic presentation, and the patient may initially be dazed with narrowed attention; disorientation is not uncommon as a result.

Adjustment disorder (case 3) is a condition that refers to the psychological reactions arising in relation to adapting to new circumstances and occurs in someone who has been exposed to a psychosocial stressor like divorce, separation etc, which is not catastrophic in nature. The usual presentations include anxiety, depression, poor concentration, irritability, anger, etc. with physical symptoms caused by autonomic arousals such as tremor and palpitations. The onset is more gradual than that of acute stress reaction, with course more prolonged. Onset must be within one month of the stress event according to ICD and three months in DSM, with some psychosocial dysfunction being required for the diagnosis to be made.

The correct answer is: A patient with long-standing anxiety describes living on the edge all his life. He has recently deteriorated. He is performing poorly at work. → Generalised Anxiety Disorder, A 42-year-old mother with no previous psychiatric history presents with confusion and highly anxious. Her son developed an acute neurological illness and had been hospitalised since the previous day. → Acute stress reaction, A 45 year-old-woman is noted to suffer from anxiety, depression, poor concentration, irritability and is unable to work for last four weeks. She divorced her husband recently. → Adjustment disorder

Question **40**

Not answered

Marked out of 8.00

Flag question

Diagnostic features of psychotic disorders

For each of the following diagnosis choose the most characteristic diagnostic feature that aids in clinical diagnosis.

Paranoid schizophrenia — Choose...

Persistent delusional disorder — Choose...

Hebephrenic schizophrenia — Choose...

Choose...
Symptoms last for only two weeks
Apathy
Prominent delusions and hallucinations
Behavioural and personality changes
Relatively preserved functioning
Amnesia
Young adult or adolescent onset
Catatonic symptoms
Prominent somatisation

Frontotemporal dementia — Choose...

Paranoid schizophrenia: Prominent delusions and hallucinations are the most prominent features of F20 paranoid schizophrenia; others in the list are mostly supplementary. Though the age of onset is not a criterion for paranoid schizophrenia (unlike hebephrenia), it is a well-known clinical observation that paranoid schizophrenia has a later age of onset compared to hebephrenia or catatonia.

A persistent delusional disorder is characterised by a later age of onset (mean 40yrs), and symptoms must be present for more that three months for a diagnosis to be made. Social functioning is somewhat preserved when compared to schizophrenia (according to DSM).

Hebephrenic schizophrenia is characterised by emotional disturbances (incongruence or a fatuous affect), abnormal premorbid personality and a younger age of onset.

The correct answer is: Paranoid schizophrenia → Prominent delusions and hallucinations, Persistent delusional disorder → Relatively preserved functioning, Hebephrenic schizophrenia → Young adult or adolescent onset, Frontotemporal dementia → Behavioural and personality changes

Question **41**

Not answered

Marked out of 3.00

Syndromes in psychopathology

Find one most appropriate diagnosis/eponym each for the syndromes described below:

A 73-year-old gentleman who has been grossly neglecting himself, his flat being untidy and filthy, hoards up objects of no practical value. — Choose...

A 46-year-old man is convinced that his skin is infested with multiple, tiny mites and insects. — Choose...

Mrs. Zakaria is a 30-year-old woman who has recently become pregnant. Her husband has developed morning sickness, abdominal pain and unusual food cravings after he found out that she was pregnant. — Choose...

Choose...
Capgras syndrome
Fregoli syndrome
Formication
Cotard syndrome
Ganser syndrome
Dhat syndrome
Couvade syndrome
De Clerambault syndrome
Ekbom syndrome
Folie a Deux
Diogenes syndrome

Explanation:

Diogenes syndrome is characterised by the hoarding of objects, usually of no practical use, and neglect of one's home or environment. It may be a behavioural manifestation of an organic disorder, schizophrenia, a depressive disorder, or OCD, or may reflect a reaction late in life to stress in a certain type of personality.

Ekbom syndrome is a monosymptomatic delusional disorder where the core delusion is a delusion of infestation. Patients believe that their skin is infected with multiple tiny, mite-like animals. It is also seen in acute confusional states (particularly secondary to drug or alcohol withdrawal), in schizophrenia, in dementing illnesses, and as delusional elaboration of tactile hallucinatory experiences.

Couvade syndrome is a conversion symptom seen in partners of expectant mothers during their pregnancy. Symptoms vary, but mimic pregnancy symptoms. It is NOT delusional in nature – the affected individual does not believe that they are pregnant.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 105-109.

The correct answer is: A 73-year-old gentleman who has been grossly neglecting himself, his flat being untidy and filthy, hoards up objects of no practical value. → Diogenes syndrome, A 46-year-old

man is convinced that his skin is infested with multiple, tiny mites and insects. → Ekbom syndrome, Mrs. Zakaria is a 30-year-old woman who has recently become pregnant. Her husband has developed morning sickness, abdominal pain and unusual food cravings after he found out that she was pregnant. → Couvade syndrome

Question **42**

Not answered

Marked out of 6.00

Choose...
Urea and electrolytes
ECG
SPECT Scan
EEG
MRI Scan
CT scan
Chest radiography
Mid stream urine
Thyroid function tests
Liver function tests

Investigations in the elderly

For each of the following clinical conditions below, match the correct investigations to be performed from the given list.

77-year-old lady admitted to the ward with a history of an acute confusional state. The patient passed urine several times at nighttime and was foul smelling. — Choose...

79-year-old lady who lives on her own admits to drinking three glasses of wine everyday when CPN visited her. She is admitted to the ward for depression and alcohol misuse. — Choose...

83-year-old lady with a history of diabetes and hypertension was admitted to the ward with a history of gradual intellectual decline, depression and cognitive impairment. She is unsteady on her feet and is also dysarthric. — Choose...

77-year-old lady who is on lithium augmentation for her depressive symptoms is constipated, lethargic and has gained weight despite poor appetite in the last six months. — Choose...

Case 1 is suggestive of a woman possibly suffering from urinary tract infection. Full blood count (Leucocytosis may indicate the seriousness of the UTI) and mid-stream urine are investigations of choice.

Case 2 refers to chronic alcoholism. Raised Gamma-glutamyl transferase (GGT) and raised mean corpuscular volume (MCV) are important features seen in alcoholic subjects.

Case 3 refers to vascular dementia (Binswanger's disease). MRI is the investigation of choice for patients with vascular dementia, subcortical type.

Case 4 describes a patient who has developed hypothyroidism, which occurs on long-term treatment with Lithium carbonate. Abnormal thyroid function test, especially raised TSH levels, is seen in patients with lithium-induced hypothyroidism.

The correct answer is: 77-year-old lady admitted to the ward with a history of an acute confusional state. The patient passed urine several times at nighttime and was foul smelling. → Mid stream urine, 79-year-old lady who lives on her own admits to drinking three glasses of wine everyday when CPN visited her. She is admitted to the ward for depression and alcohol misuse. → Liver function tests, 83-year-old lady with a history of diabetes and hypertension was admitted to the ward with a history of gradual intellectual decline, depression and cognitive impairment. She is unsteady on her feet and is also dysarthric. → MRI Scan, 77-year-old lady who is on lithium augmentation for her depressive symptoms is constipated, lethargic and has gained weight despite poor appetite in the last six months. → Thyroid function tests

Question **43**

Not answered

Marked out of

Common features of personality disorders

Choose one option from the list above for each question below.

Choose...
Obsessive-compulsive
Paranoid
Dependent
Antisocial
Borderline
Histrionic
Narcissistic
Avoidant
Schizoid
Schizotypal

A 28-year-old man has complained to local council numerous times about his neighbours throwing rubbish into his garden. Being vengeful, he emptied his weekly rubbish collection into their backyard before going in person to the council office to make a complaint. He has always had trouble with different neighbours and has changed nine houses in last three years. Choose...

A 34-year-old lady who is attending outpatient clinic regularly becomes angry when asked to see a registrar during a visit, as the consultant was on holiday. She swears vehemently at the receptionist and kicks a flower vase in the waiting room. She threatens to kill herself if the consultant does not see her. Choose...

A 37-year-old lady believes in clairvoyance and sixth sense. She and her sister belong to an occult group, which believes in communicating with 'medium' to enhance one's spirit. She collects carcasses of animals that die on country roads to preserve their hides. Choose...

A 41-year-old man insists that his GP refer him to a psychiatric consultant. He presented with depression but has not responded to initial pharmacotherapy. He has a personal history of financial bankruptcy. He maintains that all his seven children born to his divorced wife are in very high positions, and they would not want to see him 'being so down.' Choose...

Explanation:

Clinical features of patients with paranoid personality disorder include suspicions that others are exploiting, harming, or deceiving them; reading hidden demeaning or threatening meanings into benign remarks/events; holding long-standing grudges; believing that others are not loyal or trustworthy; reluctance to confide in others; feeling others are attacking their character or reputation; having recurrent suspicions of infidelity.

Clinical features of patients with borderline personality disorder include unstable self-image; unstable interpersonal relationships; impulsive behaviour in areas such as personal safety and substance misuse; self-harm, suicidal threats or gestures; affective instability due to marked reactivity of mood; episodes of micro-psychosis; chronic feelings of emptiness; frantic efforts to avoid real or imagined abandonment; inappropriate, intense anger.

Schizotypal disorder is classified along with schizophrenia and related disorders in ICD-11, but along with cluster A/'odd-eccentric' personality disorders in DSM-5. It shares some of the same features of schizophrenia, but not the delusions or hallucinations. Symptoms include ideas of reference, excessive social anxiety, odd beliefs or magical thinking, unusual perceptions (e.g. illusions), odd/eccentric behaviour or appearance, no close friends/confidants, odd speech, inappropriate or constricted affect, and suspiciousness.

Persons with narcissistic personality disorder have a grandiose sense of self-importance; they consider themselves special and expect special treatment. They handle criticism poorly and may become enraged when someone dares criticize them, or they may appear utterly indifferent to criticism. They are commonly interpersonally exploitative, and their self-esteem is fragile. They use primitive defence mechanisms, including projection which involves attributing one's own internal unacceptable ideas and impulses to an external target, such as another individual, and reacting accordingly to them.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 562-565.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 228-229.

The correct answer is: A 28-year-old man has complained to local council numerous times about his

neighbours throwing rubbish into his garden. Being vengeful, he emptied his weekly rubbish collection into their backyard before going in person to the council office to make a complaint. He has always had trouble with different neighbours and has changed nine houses in last three years. → Paranoid, A 34-year-old lady who is attending outpatient clinic regularly becomes angry when asked to see a registrar during a visit, as the consultant was on holiday. She swears vehemently at the receptionist and kicks a flower vase in the waiting room. She threatens to kill herself if the consultant does not see her. → Borderline, A 37-year-old lady believes in clairvoyance and sixth sense. She and her sister belong to an occult group, which believes in communicating with 'medium' to enhance one's spirit. She collects carcasses of animals that die on country roads to preserve their hides. → Schizotypal, A 41-year-old man insists that his GP refer him to a psychiatric consultant. He presented with depression but has not responded to initial pharmacotherapy. He has a personal history of financial bankruptcy. He maintains that all his seven children born to his divorced wife are in very high positions, and they would not want to see him 'being so down.' → Narcissistic

Question **44**

Not answered

Marked out of 4.00

Childhood & developmental disorders

For each of the following vignette below choose one correct answer from the given list:

A 7-year-old boy is frequently skipping school with complaints of tummy ache, diarrhea, back pain and tiredness. He is increasingly refusing to sleep on his own and wants to share his parents' bedroom. He does not want his mum to leave him at home and go out on her own as he is scared of 'black monsters' in the house.

A family with three children aged 11, 15 and 18 has recently migrated to Leeds from London. The 11-year-old boy has developed troublesome stomach ache and back pain since the relocation. The symptoms ease off during the day if he stays at home taking bed-rest. The GP could not find any physical cause. He seems free of these problems over the weekends.

A 15-year-old girl has changed a lot in last six months according to her mother. She isolates herself in her room most of the time, talking to few selected friends on her mobile phone most of the time. She spends much time on the internet chat rooms and social networks. She has little conversation if any with her 8-year-old brother and her parents, though her academic progress has been satisfactory.

A 14-year-old girl with anhedonia and poor concentration, poor appetite, and insomnia lacks motivation to do anything and is participating poorly during activities at her school.

Choose...
School refusal
Juvenile delinquency
Depressive episode
Schizophrenia - childhood onset
Normal adolescence
ADHD
Truancy
Separation anxiety disorder
Conduct disorder
Pervasive developmental disorder
Disintegrative disorder
Bipolar depression

Multiple somatic complaints in children is most often due to an internalizing emotional problem that can present as school refusal (case 2). In separation anxiety (case 1), school refusal is present along with an anxiety of leaving the caregiver. The latter is often not overtly described, but various reasons such as fear of dark or monsters are invoked to ensure the presence of the caregiver in proximity.

It is normal to stay aloof from family and indulge in one's age-specific interests during adolescence (case 3).

Major depression is the most severe condition in adolescents, with either "sad or irritable mood, or

anhedonia, along with at least five other symptoms, such as social withdrawal, worthlessness, guilt, suicidal thoughts or behaviour, sleep increase or decrease, decreased motivation and/or concentration, and increased or decreased appetite" (DSM-5). A crucial distinction between depression as an illness and the normal mood variations of childhood and adolescence is the association of functional impairment with depression, in addition to a lack of responsiveness of the depressed mood to contextual changes.

The correct answer is: A 7-year-old boy is frequently skipping school with complaints of tummy ache, diarrhea, back pain and tiredness. He is increasingly refusing to sleep on his own and wants to share his parents' bedroom. He does not want his mum to leave him at home and go out on her own as he is scared of 'black monsters' in the house. → Separation anxiety disorder, A family with three children aged 11, 15 and 18 has recently migrated to Leeds from London. The 11-year-old boy has developed troublesome stomach ache and back pain since the relocation. The symptoms ease off during the day if he stays at home taking bed-rest. The GP could not find any physical cause. He seems free of these problems over the weekends. → School refusal, A 15-year-old girl has changed a lot in last six months according to her mother. She isolates herself in her room most of the time, talking to few selected friends on her mobile phone most of the time. She spends much time on the internet chat rooms and social networks. She has little conversation if any with her 8-year-old brother and her parents, though her academic progress has been satisfactory. → Normal adolescence, A 14-year-old girl with anhedonia and poor concentration, poor appetite, and insomnia lacks motivation to do anything and is participating poorly during activities at her school. → Depressive episode

Question **45**

Not answered

Marked out of 3.00

Flag question

Classification of disorders in the elderly

For each of the following clinical conditions below, match the associated psychiatric syndrome from the given list:

A 77-year-old widowed gentleman became increasingly isolated over the last two years and started to neglect himself. He lives in filthy, poor conditions. Although necessitating intervention, he was resistant to help.

A 78-year-old lady with hearing impairment, and who lives on her own, believes that the neighbours are spying on her. She can also hear neighbours talking about her all the time through the walls.

An 87-year-old lady with a history of falls and was brought to the A&E by the paramedics. She is agitated, paranoid, confused and also has visual hallucinations. She was previously reported to be a high functioning lady.

Choose...
Chronic confusional state
Old age depression
Adjustment disorder
Very-late-onset schizophrenia-like psychosis
Acute confusional state
Charles Bonnet syndrome
Hypomania
Diogenes syndrome
Pseudodementia
Korasakoff psychosis

Explanation:

Diogenes syndrome is characterised by the hoarding of objects, usually of no practical use, and neglect of one's home or environment. It may be a behavioural manifestation of an organic disorder, schizophrenia, a depressive disorder, or OCD, or may reflect a reaction late in life to stress in a certain type of personality.

Very-late-onset schizophrenia-like psychosis (VLOS) has onset after 60 years of age. Risk factors for VLOS include female gender, being from an immigrant population, greater abnormalities in brain

structures, family history of schizophrenia or avoidant personality, paranoid or schizoid personality disorder, hearing loss, and being from a lower socioeconomic status. VLOS has female preponderance, lower genetic risk (lower incidence of family history of psychosis), and higher rates and more severe paranoid symptoms and persecutory delusions. Partition delusions (concerning the unwanted permeability of borders) are highly prominent.

Delirium (or acute confusional state) is an acute-onset neuropsychiatric syndrome, marked by changes to cognitive domains (e.g. attention and memory) and wider behavioural changes (e.g. sleep-wake cycle and motor disturbances), occurring in the context of medical illness.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 108.

Tampi RR et al. Psychotic disorders in late life: a narrative review. Therapeutic advances in psychopharmacology. 2019 Oct;9:1-13.

Suen YN et al. Late-onset psychosis and very-late-onset-schizophrenia-like-psychosis: an updated systematic review. International Review of Psychiatry. 2019 Aug 18;31(5-6):523-42.

Van Assche L et al. The neuropsychology and neurobiology of late-onset schizophrenia and very-late-onset schizophrenia-like psychosis: a critical review. Neuroscience & Biobehavioral Reviews. 2017 Dec 1;83:604-21.

FitzGerald JM, Price A. Delirium in the acute hospital setting: the role of psychiatry. BJPsych Advances. 2021 Jan;27(1):49-59.

The correct answer is: A 77-year-old widowed gentleman became increasingly isolated over the last two years and started to neglect himself. He lives in filthy, poor conditions. Although necessitating intervention, he was resistant to help. → Diogenes syndrome, A 78-year-old lady with hearing impairment, and who lives on her own, believes that the neighbours are spying on her. She can also hear neighbours talking about her all the time through the walls. → Very-late-onset schizophrenia-like psychosis, An 87-year-old lady with a history of falls and was brought to the A&E by the paramedics. She is agitated, paranoid, confused and also has visual hallucinations. She was previously reported to be a high functioning lady. → Acute confusional state

Question **46**

Not answered

Marked out of 3.00

Drug misuse and clinical presentations

Choose one option for each question below

A 22-year-old university student develops intense pounding headaches, marked fatigue anxiety and nausea immediately after his exams. He denies using alcohol or smoking. Choose...

A 30-year-old man has panic attacks and recurrent cough. He has a red eye on examination but has no needle marks. He appears quite apathetic and unconcerned. Choose...

A 44-year-old truck driver presents with intense paranoia and anxiety; he has tachycardia and reports having ants crawling under his clothes. Choose...

Choose...
Cocaine intoxication
Cocaine withdrawal
Caffeine withdrawal
Cannabis withdrawal
Cannabis intoxication
Opiate intoxication
Korsakoff psychosis
Amphetamine use
Opiate withdrawal

Caffeine withdrawal is not recognised in DSM but is a likely diagnosis in a student who has just appeared for an exam (case 1). DSM research criteria lists headache, marked fatigue or drowsiness, marked anxiety or depression, nausea or vomiting as the prominent features that can be seen even at low doses (from 100mg/day), beginning within 12 hours, peaking at 24-48 hours and lasting up to 1 week.

Cannabis intoxication (case 2) is characterised by impaired motor coordination, euphoria, sensation of slowed time, social withdrawal, conjunctival injection (red eye), increased appetite (munchies), dry mouth and tachycardia. Impairment in motor skills remains after the euphoriant effects are gone.

Some patients (case 3) under cocaine intoxication report a sensation of bugs crawling beneath the skin (formication). It is associated with increased energy, increased confidence, heightened self-esteem, and perceived improvement on mental and physical tasks, euphoria, and diminished need for sleep. With high doses agitation, irritability, impaired judgment, impulsive aggression, some symptoms of mania with associated tachycardia, hypertension, and mydriasis are noted.

The correct answer is: A 22-year-old university student develops intense pounding headaches, marked fatigue anxiety and nausea immediately after his exams. He denies using alcohol or smoking. → Caffeine withdrawal, A 30-year-old man has panic attacks and recurrent cough. He has a red eye on examination but has no needle marks. He appears quite apathetic and unconcerned. → Cannabis intoxication, A 44-year-old truck driver presents with intense paranoia and anxiety; he has tachycardia and reports having ants crawling under his clothes. → Cocaine intoxication

Question **47**

Not answered

Marked out of 4.00

Flag

Supportive techniques while interviewing

For each of the following examples, identify the type of supportive techniques aimed at eliciting information

Let's plan on working on this together — Choose...

I would like to offer whatever help I can — Choose...

I can see that you are finding it difficult to tell me about your relationships. It is fine, and we can come back to this when you feel ready to discuss this with me — Choose...

In the past, you have managed this difficult situation very well, and I think you can manage it again. — Choose...

Choose...
Acknowledgment of affect
Partnering
Encouragement
Reassurance
Validation
Postponement
Advice
Statement of respect
Support

Partnering: The interviewer encourages the patient to ask questions and to express any concerns, encouraging team working. Support: Emphasising the interviewer's role in helping. Postponement: Conscious and deliberate postponement of delicate issues; but must be opened at an appropriate time. Encouragement: to inspire with the courage or confidence to do something.

The correct answer is: Let's plan on working on this together → Partnering, I would like to offer whatever help I can → Support, I can see that you are finding it difficult to tell me about your relationships. It is fine, and we can come back to this when you feel ready to discuss this with me → Postponement, In the past, you have managed this difficult situation very well, and I think you can manage it again. → Encouragement

Question **48**

Not answered

Marked out of 4.00

Flag

Defining defence mechanisms

For each of the definitions given below, identify the most likely defence mechanism from the list provided.

Unconscious attribution of one's attitudes and urges to other persons due to the painful affect aroused. — Choose...

Unconscious transformation of unacceptable impulses into exactly opposing attitudes, impulses, feelings or behaviours. — Choose...

Expression of an unconscious impulse through an action, thereby gratifying the impulse. — Choose...

Choose...
Denial
Isolation
Acting out
Projection
Repression
Splitting
Sublimation
Introjection
Reaction formation
Identification with the aggressor

Unconscious internalisation of the qualities of an object or person. Choose...

Explanation:

Projection is a primitive or immature defense mechanism that involves attributing one's own internal unacceptable ideas and impulses to an external target, such as another individual, and reacting accordingly to them.

Reaction formation involves externally expressing attitudes and behaviours that are the opposite of the unacceptable internal impulses (e.g. being extra polite to a person to avoid expressing anger towards them).

In acting out, patients directly express unconscious wishes or conflicts through action to avoid being conscious of either the accompanying idea or the affect. Tantrums, apparently motiveless assaults, and pleasureless promiscuity are typical examples.

The introjection of a loved one involves the internalisation of characteristics of the object with the goal of ensuring closeness to and constant presence of the object. If the object is lost, introjection nullifies or negates the loss by taking on the characteristics of the object, thus in a sense internally preserving the object.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 892-893.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 577, 1066.

The correct answer is: Unconscious attribution of one's attitudes and urges to other persons due to the painful affect aroused. → Projection, Unconscious transformation of unacceptable impulses into exactly opposing attitudes, impulses, feelings or behaviours. → Reaction formation, Expression of an unconscious impulse through an action, thereby gratifying the impulse. → Acting out, Unconscious internalisation of the qualities of an object or person. → Introjection

Question **49**

Not answered

Marked out of

Motor disorders in psychiatric clinic

Choose one option for each of the following descriptions

Jerky movements that can be voluntarily suppressed for a brief period and are habitual — Choose...

Shock-like muscle contractions, which can be local or generalised — Choose...

Rhythmic alternating contractions of agonist and antagonist muscles occurring with arms outstretched — Choose...

Violent flinging movement of half of the body — Choose...

Choose...
Pill rolling tremor
Tardive dyskinesia
Myoclonus
Tics
Intention tremor
Chorea
Rigidity
Stupor
Hemiballismus
Dystonia

Explanation:

Tics are spontaneous, repetitive, rhythmic movements that can be motor or vocal. They are semi-voluntary and can only be resisted for a short time with difficulty.

Myoclonus consists of generalised or focal, irregular, rapid jerks. It differs from most movement disorders in several respects. Myoclonus originates in abnormal discharges from motor neurons in the cerebral cortex, brainstem, or spinal cord rather than the basal ganglia. It may persist when patients are asleep or comatose. Also, unlike the spontaneous movements of chorea or Parkinson disease, myoclonic jerks can be triggered by voluntary movements or external stimuli, such as a noise, touch,

or light.

A resting tremor occurs in a body part that is relaxed and completely supported against gravity (e.g., when resting on the arm of a chair). Approximately 70% of patients with Parkinson disease have resting tremor as the presenting feature. The classic parkinsonian tremor begins as a low-frequency, pill-rolling motion of the fingers, progressive to forearm pronation/supination and elbow flexion/extension. It is typically unilateral, occurs at rest, and fades with voluntary movement. It can re-emerge after latency with maintenance of posture, which can create diagnostic confusion.

Hemiballismus is characterised by involuntary, large-scale 'throwing' movements of one limb or one body side.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 112, 132.

Kaufman DM, Geyer HL, Milstein MJ, Rosengard JL. Kaufman's Clinical Neurology for Psychiatrists, 9th Edition. 2023. p. 444.

Crawford III PF, Zimmerman EE. Tremor: sorting through the differential diagnosis. American family physician. 2018 Feb 1;97(3):180-6.

The correct answer is: Jerky movements that can be voluntarily suppressed for a brief period and are habitual → Tics, Shock-like muscle contractions, which can be local or generalised → Myoclonus, Rhythmic alternating contractions of agonist and antagonist muscles occurring with arms outstretched → Pill rolling tremor, Violent flinging movement of half of the body → Hemiballismus

Question **50**

Not answered

Marked out of 4.00

Flag

Clinical rating scales

For each description given below, select the relevant rating scale from the list above;

This scale is useful to assess the severity of various mental illnesses — Choose...

11 item observer scale used in mania — Choose...

Clinician administered rating scale for assessment of severity and monitoring the change of symptoms in patients with schizophrenia — Choose...

19-item test to assess cognitive functions — Choose...

Choose...
Yale-Brown obsessive compulsive scales (YBOCS)
Eysenck personality questionnaire (EPQ)
Positive and negative symptoms scale (PANSS)
Clinical Global impressions scale (CGI)
Young mania rating scale
Mini mental state examination (MMSE)
General health questionnaire (GHQ)
Minnesota multiphasic personality inventory (MMPI)
Brief psychiatric rating scale (BPRS)
Hospital anxiety and depression scale (HADS)

Explanation:

The Brief Psychiatric Rating Scale (BPRS) was developed in the late 1960s as a short clinician-rated scale (based on observation) for measuring the severity of psychiatric symptomatology. It consists of 18 or 24 items, each rated on a 7-8 point scale. It covers a broad range of areas, including thought disturbance, emotional withdrawal and retardation, anxiety and depression, and hostility and suspiciousness.

The Young Mania Rating Scale (YMRS) has 11 items and is based on the patient's subjective report of their condition over the previous 48 hours. Additional information is based upon clinical observations made during the course of the clinical interview. There are four items that are graded on a scale from 0 to 8 (irritability, speech, thought content, and disruptive/aggressive behaviour), while the remaining seven items are graded on a scale of 0 to 4. Mania is indicated by a score of 12, but many clinical studies require YMRS > 20 to determine a manic state

The Positive and Negative Symptom Scale (PANSS) is a clinician-administered rating scale for the

assessment of severity and monitoring of change of symptoms in patients with a diagnosis of schizophrenia. It contains items that cover positive symptoms, negative symptoms, and general psychopathology.

The MMSE is a 19-item, 30-point cognitive test developed in the mid-1970s to provide a bedside assessment of a broad array of cognitive functions, including orientation, attention, memory, construction, and language.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 34-38, 45.

Di Giacomo E et al. Unblending borderline personality and bipolar disorders. Journal of psychiatric research. 2017 Aug 1;91:90-7.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 97.

The correct answer is: This scale is useful to assess the severity of various mental illnesses → Brief psychiatric rating scale (BPRS), 11 item observer scale used in mania → Young mania rating scale, Clinician administered rating scale for assessment of severity and monitoring the change of symptoms in patients with schizophrenia → Positive and negative symptoms scale (PANSS), 19-item test to assess cognitive functions → Mini mental state examination (MMSE)

Finish review

PREVIOUS ACTIVITY
Rating Scales

NEXT ACTIVITY
EMIs Assessment Part 2

Jump to...

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